

ONTARIO
SUPERIOR COURT OF JUSTICE

B E T W E E N:

PHILLIPE DESBIENS and CECILE
DESBIENS

Plaintiffs

- and -

MICHAEL MORDINI and MARIA
LUONGO ROSS WEMP LEASING INC.

Defendants

)
)
) Neil Wheeler, for the Plaintiff
)
)
)

)
)
) Cameron Godden, for the Defendants
)
)
)

)
) Dan Guttman, for the Attorney General of
) Ontario, Intervenor
)
)

) **HEARD:** February 18, 19, 20, 23, 24, 25,
26, 27, March 1, 2, 3, 4, 5, 8, 9, 10, 11, 12,
16, 17, 18, 19, 23, 24, 25, 26, 29, 30, 31,
April 2, 14, 16, May 18, 19, 20

H. SPIEGEL J.

REASONS FOR JUDGMENT

Overview.....	3
Factual background.....	4
Mr. Desbiens' Medical Condition Before the 1986 Accident	5
The 1986 Accident.....	6
Mr. Desbiens Medical Condition After the 1986 Accident	6
The Car Accident.....	16
Treatment and Medical Condition After the Car Accident.....	16
Level of Functioning After the Car Accident	22

Conclusion	27
The Legislative Scheme	27
Overview of the Guides	30
Catastrophic Impairment.....	32
Position of the Plaintiffs.....	32
Did Mr. Desbiens Sustain a Catastrophic Impairment as Defined in Clause (g)?	33
Conclusion	38
Did Mr. Desbiens Sustain a Catastrophic Impairment as Defined in Clause (f)?.....	38
Conclusion	52
Mr. Desbiens' Impairments Superimposed On His Pre-Existing Paraplegia	52
Dr. Delaney's Approach	53
Dr. Ameis Approach	55
Analysis.....	56
Conclusion	60
On a proper interpretation of the Regulation is it in accordance with the Guides to assign percentage ratings to Mr. Desbiens' psychological impairments and combine them with his physical impairments, for the purpose of determining whether his impairments meet the definition of catastrophic as defined by clause (f)?	61
Analysis.....	64
Statutory Interpretation	64
Purpose of the Legislation	66
If his psychological impairments are combined with his physical impairments do Mr. Desbiens' impairments meet the definition of catastrophic as defined by clause (f)?	76
Cross-examination of Dr. Ameis On Prior Negative Judicial Comments	77
Assessment of Damages	79
Non-pecuniary General Damages	79
<i>Family Law Act</i> Damages of Ms Desbiens	82
Section 61(2)(e)- Care Guidance and Companionship	82
Section 61(2)(d) - Reasonable Allowance for the Value of Nursing, Housekeeping or Other Services	83
Special Damages	86
Future Care.....	91
Housing	92
Attendant Care	96
Other Attendant Care Needs	101
Other Activities of Daily Living Costs	101
Discount Rate.....	107
Contingencies.....	107
Life Expectancy and Actuarial Evidence.....	110
Summary of Future Care Costs.....	111
Deductions for SABs Received	112
Summary of Damage Assessment	112
Gross-up.....	112

Overview

[1] The gods of good fortune have not been kind to Phillippe Desbiens.

[2] In 1986, at the age of 44, he was rendered a paraplegic due to an injury to his spinal cord when he fell off a roof in the course of his employment (the “1986 accident”). Ironically it was the buckle of his safety harness that caused the injury to his spine.

[3] On November 8, 1999, while operating his wheelchair on a sidewalk, Mr. Desbiens was struck by a motor vehicle driven by the defendant Mordini and owned by the defendants Maria Luongo and Ross Wemp Leasing Inc. (the “car accident”). As a result of the collision Mr. Desbiens was thrown approximately 10 feet into the roadway. He suffered a displaced fracture of the right femur and a variety of other soft tissue injuries to the right side of his body.

[4] The plaintiffs claim damages from the defendants as a result of injuries sustained by the plaintiff Mr. Desbiens. The defendants have admitted liability for the accident and have admitted that the injuries sustained meet the threshold requirement contained in s. 267.5(5) of the *Insurance Act*,¹ as being a permanent serious impairment of an important physical, mental, or psychological function (the “permanent serious threshold”).

[5] Mr. Desbiens alleges that before the car accident he was able to function independently with little help from others but since the car accident he is dependent on others in most of his activities of daily living. As a result he claims that he has incurred, and will continue to incur, substantial attendant care and other healthcare expenses.

[6] Had the car accident occurred 3 years and 8 days earlier it is likely that Mr. Desbiens’ claim would have been resolved without the necessity of a trial because at that time the *Act* barred all claims for pecuniary loss but there were very generous benefits available on a no fault basis for medical, rehabilitation and attendant care costs. However, on November 1, 1996, Bill 59 came into force so that at the time of the car accident, the combined effect of sections

267.5(3) and (4) of the *Act* protected the defendants from any claim for expenses for health care unless Mr. Desbiens sustained a catastrophic impairment as defined in subsection 5(1) of Ontario Regulation 461/96 (the “Regulation”).

[7] It is the plaintiffs’ position that Mr. Desbiens sustained a catastrophic impairment as defined in clauses (f) and/or (g) of subsection 5(1) of the Regulation (“clauses (f) and (g)”), which are reproduced here:

5(1) For the purposes of subsection 267.5(4) of the *Act*, “catastrophic impairment” means,

- (f) subject to subsections (2) and (3), any impairment or combination of impairments that, in accordance with the American Medical Association’s *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993, results in 55 per cent or more impairment of the whole person;
- or
- (g) subject to subsections (2) and (3), any impairment that, in accordance with the American Medical Association’s *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993, results in a class 4 impairment (marked impairment) or class 5 impairment (extreme impairment) due to mental or behavioural disorder.

[8] Alternatively, the plaintiffs challenge the constitutional validity of clauses (f) and (g) on the grounds that they discriminate against persons like Mr. Desbiens on the basis of a mental or physical disability.

[9] The defendants take the position that Mr. Desbiens did not sustain a catastrophic impairment as defined in clauses (f) and/or (g) and that the Regulation does not offend the *Charter*.

Factual background

[10] Mr. Desbiens was born in Quebec on December 7, 1942. He was one of 11 children. He achieved a grade five education in Quebec. When he was 15 years old he moved with his

¹ R.S.O. 1990 C. 1.8 as amended (the “*Act*”).

parents to northern Ontario where he worked in a variety of labouring jobs. He did not attend school in Ontario because of his limited English.

[11] He married in his early twenties but after a short time his wife obtained an annulment. He met his present wife, Cecile in 1970 and they have been together ever since. They married in 1999 shortly before the car accident. They have no children of their union but Ms Desbiens has seven children by a previous marriage.

Mr. Desbiens' Medical Condition Before the 1986 Accident

[12] In 1971 the Desbiens moved to Mississauga where Mr. Desbiens obtained work in the roofing trade. After a few years he started his own roofing company, which he says was quite successful.

[13] In 1975, Mr. Desbiens was involved in a car accident in which he sustained a whiplash injury to his neck. He was examined in hospital but not admitted. He experienced upper back and neck discomfort. He was off work for about a year, which he says was because he didn't have a truck because it had been damaged in the accident. After that case was settled he bought another truck and returned to his roofing business.

[14] Dr. Lee has been Mr. Desbiens' family physician since November 20, 1981. Dr. Lee was called as a witness on behalf of the plaintiffs and his clinical notes and records were made an exhibit at trial.

[15] Between November 1981 and the 1986 accident Mr. Desbiens saw Dr. Lee on approximately 17 occasions, the vast majority of which were in 1981 and 1982 when he complained of neck pain and stiffness attributed to the 1975 car accident.

[16] In May 1985 Mr. Desbiens sprained his back as a result of lifting a heavy object but, according to Dr. Lee, appeared to recover quickly from this injury. He also had a broken ankle that gave him some problems for approximately 6 months. In the 3 years prior to the 1986

accident Mr. Desbiens had only seen Dr. Lee on four occasions and had not seen him at all for more than one year prior to the 1986 accident.

[17] Dr. Lee testified that Mr. Desbiens' overall health was pretty good prior to the 1986 accident. I accept Dr. Lee's evidence in this regard.

The 1986 Accident

[18] On June 25, 1986, while working as a roofer, Mr. Desbiens slipped on a roof and sustained a fracture of the T11-T12 vertebrae as a result of which he was rendered paraplegic at the T11-12 level. He was first taken to the Mississauga Hospital and then transferred one week later to the Toronto Western Hospital where he underwent surgery for fusion of his spine and the installation of Harrington rods in his back. On July 17, 1986 he was discharged from the Western Hospital and admitted to the Lyndhurst Hospital for rehabilitation. He tried to get around on crutches for a period of time but because of back pain he had to rely on a wheelchair for mobility. While at Lyndhurst he developed a deep venous thrombosis, which at the time of his discharge, was quiescent.

[19] Mr. Desbiens admitted that while at Lyndhurst he felt depressed about his condition and anxious about how he would adapt to it. However, before his discharge he was taught skills to help him manage at home independently. After being at home for a while he realized that these skills made it possible for him to take care of himself and this made him feel more comfortable with his situation which relieved his anxiety.

Mr. Desbiens Medical Condition After the 1986 Accident

[20] Mr. Desbiens testified that between the 1986 accident and the car accident he had low back pain perhaps two or three times and always for a short duration. Furthermore, he testified to having neck pain once or twice but really could not remember how many times. He said he had some pains in his left shoulder before the 1986 accident and once after the accident. He further testified to having "regular headaches" which he attributed to his sinuses but he said the

headaches did not stop him from doing what he had to do. He did not recall having any problems with his left elbow prior to the accident, but said that he had a sore right wrist while at Lyndhurst. He testified that he did have some episodes of dizziness “a few times.” He stated that prior to the car accident his medications consisted of 1 mg. of Ativan per day, 3 Tylenol #3 per day and Metamucil to assist him with his bowel movement.

[21] Ms Norlock, Ms Desbiens’ granddaughter, testified that she could not recall Mr. Desbiens having any health complaints before the car accident other than with respect to his gall bladder.

[22] Ms Desbiens testified that prior to the car accident Mr. Desbiens did complain of neck pain but nothing else.

[23] The first time that Dr. Lee saw Mr. Desbiens after the 1986 accident was on February 27, 1987. From that time until the car accident Dr. Lee saw Mr. Desbiens on approximately 100 occasions. Dr. Lee’s records and his testimony at trial demonstrate that between February 27, 1987 and the car accident Mr. Desbiens presented with at least 20 complaints of low back pain which were treated with Tylenol #3 as well as a variety of muscle relaxants. On average Dr. Lee prescribed 100 Tylenol #3 per month to be taken at the rate of no more than 2 every four hours. On cross examination Dr. Lee conceded he would consider the low back pain as chronic since it persisted for more than 12 months.

[24] On March 5, 1997 Mr. Desbiens reported to Dr. Lee that he was experiencing quite severe headaches in the occipital frontal region on a daily basis. On May 5, 1997 a similar complaint was made about headaches that were relieved by the use of Motrin and Tylenol #3. In May 1998 there was a similar complaint of occipital headache on an intermittent basis. There were also a small number of complaints of neck pain, dizziness and joint pain in his hands and shoulders.

[25] Mr. Desbiens had recurring urinary tract infections (UTI) a common problem for paraplegics, which was treated by antibiotics. In May 1998 he was found to have Type 2 diabetes mellitus. His blood sugar levels were controlled with proper diabetic diet.

[26] Defendants' counsel submitted that the court should find that Mr. Desbiens was not being truthful to the court or to many of the health care professionals who treated or assessed him when he described his pre-car accident state of health, and in particular when he described the frequency, severity and location of his low back pain.

[27] I agree that there is a stark contrast between Mr. Desbiens' description at trial of the frequency and severity of low back pain and the complaints that were made to Dr. Lee. However I am not prepared to find that Mr. Desbiens was deliberately attempting to mislead the court. In fairness to Mr. Desbiens, he did qualify his answers by such phrases as "hard to say", "don't recall all the time", "and something like that", "I am not sure". It must also be remembered that he was testifying about events that occurred between 1986 and 1999. As well, it is likely that his recollection of his back problems prior to the car accident was affected by his perception that since the car accident, his pain and the restrictions on his activities of daily living were much more severe. Viewed from this perspective one can understand how he may have honestly underestimated his pre car accident complaints. Neither he nor his counsel disputed the accuracy of Dr. Lee's clinical notes and where there is a conflict between Mr. Desbiens' recollection of his complaints and Dr. Lee's clinical notes I have no hesitation in finding the latter are more accurate

[28] It should also be noted that in the early part of 1999 Dr. Lee felt that the pain was in the higher region of his back and probably due to his gall bladder problems that were resolved once the gall bladder was removed in May 1999. As well, Dr. Lee noted that on July 26, 1999, the last occasion he saw Mr. Desbiens prior to the car accident, his only complaint was of dizziness. Dr. Lee testified that just before the car accident Mr. Desbiens' shoulders, elbow and arm were not causing problems. He agreed that Mr. Desbiens had headaches but that the last complaint of headache was approximately 1½ years prior to the car accident.

[29] Insofar as Mr. Desbiens may have given an inaccurate history to health professionals whose opinions were tendered in evidence the court must consider, to what extent if any, this affects the weight to be given to these opinions. However counsel for the defendants has not specifically referred to any expert opinion that he says is significantly tainted by the inaccurate history given by Mr. Desbiens.

Mr. Desbiens' Living Accommodations After the 1986 Accident

[30] At the time of the 1986 accident Mr. and Ms Desbiens lived at 3375 Pony Trail Drive, Mississauga in a one-bedroom apartment.

[31] Because the 1986 accident occurred while Mr. Desbiens was in the course of his employment, the Workers' Compensation Board² (WCB) was prepared to pay for modifications to the apartment including enlarging the bedroom and bathroom door and replacing the existing vanity in the bathroom with either a wall hung basin or a pedestal type sink. This would allow Mr. Desbiens to enter the bathroom in the wheelchair and transfer onto the toilet or onto a bath transfer seat to take a shower and to use the sink for washing and shaving. Sketches of the proposed modifications were prepared and on September 18, 1986, the WCB obtained a written estimate in the amount of \$5,635 for the cost of the modifications.

[32] Mr. Desbiens was advised by the WCB that it was necessary for him to obtain written permission from the landlord to carry out the modifications. Mr. Desbiens testified that he made inquiries of the building manager and was advised that the landlord was not prepared to consent to the modifications.

[33] Mr. Desbiens' WCB file provides conflicting evidence on this issue. At page 186 of Exhibit 10b there is a copy of a letter dated September 4, 1986 from Lynn Levesque, the property manager of the Landlord, giving permission for the repairs or alterations on condition that the apartment be restored to its original condition should Mr. Desbiens decide to move.

² Since 1998 it has been known as The Workers Safety Insurance Board (WSIB). See *Workplace Safety and Insurance Act, 1997*, S.O. 1997, c. C.16, s. 159.

Page 191 is a copy of a memo dated September 10, 1986, in which Mr. Greco of the WCB notes that he has already received the written permission from the management company of the landlord to allow the changes.

[34] Page 194 of Exhibit 10b is a memo prepared by Mr. Greco in which he notes that he attended a meeting on September 13, 1986 with Mr. Desbiens at his apartment to discuss the necessary changes. He notes that, Mr. Jack Levesque, Maintenance Manager for the building, and Mr. Lee Deman, Contractor, were present at the meeting. He also notes that Mr. Levesque indicated that the proposals for modifications do not seem unreasonable but that he will have to discuss it with the owners of the building and once he has done this he will inform the WCB in writing.

[35] At the bottom of Page 195 there is a hand written note which indicates that on September 22, 1986, Mr. Greco received a phone call from Mr. Levesque advising that the owners of the apartment would not allow any changes whatsoever to any units in the building. The note indicates that Mr. Greco requested that they confirm their position in writing. However, the file does not contain any such confirmation.

[36] In or about September 1986, apartment 307, a two-bedroom apartment with a larger bathroom became available in the same building. Mr. Desbiens testified that he liked the location of his apartment building and rather than move to a different location he was prepared to move into apartment 307. The WCB inspected the new apartment and agreed to provide a number of items that are normally provided to any wheelchair-confined individual, such as grab bars, commode, and a ramp from the living room to the balcony. At Mr. Desbiens' request the WCB also provided a microwave oven so that Mr. Desbiens could continue to cook in his own household. No further modifications were requested or provided. Mr. Desbiens testified that until he was injured in the car accident the apartment suited his needs. At the time of trial the Desbiens were still living in apartment 307.

The Evidence of Mr. & Ms Desbiens and Ms Norlock

[37] As a result of the paraplegia Mr. Desbiens had no feeling in his legs except for a small area on his right thigh. He had no voluntary control over his bowel and bladder functions. He had to catheterize himself every 4 hours in order to empty his bladder. To move his bowels he had to use his fingers to stimulate his anal area in order to induce a spasm. He did not require any assistance in his bowel or bladder routine. He was able to transfer from the wheelchair to a smaller 4-wheel chair or commode and wheel himself into the bathroom, transfer on to the toilet and lean forward and “get the job done.” Catheter supplies were delivered once a month. He was able to unpack the supplies, store them and bring them into the bedroom when required.

[38] He was required to wear anti-thrombotic stockings (TED stockings) in order to avoid an embolism. He was able to put on and remove the TED stockings without assistance. He had a sheepskin covering over the mattress of his bed to prevent bedsores. He was generally able to wash the sheepskin on his own.

[39] He could shave and shower by himself. He needed help to transfer from the commode to the shower bench but he could get out of the shower on his own.

[40] He engaged in a wide range of activities such as wheelchair basketball, polo and racing. He also did woodworking, ceramics and some home repairs. For example he installed a screen around his balcony, laid down some carpeting and together with his wife, did painting around the apartment. He was able to shop for groceries. He was able to select the grocery items and put them in a buggy attached to the front of his wheelchair and then transfer them to his van. The apartment building has laundry facilities on the ground floor and he and his wife shared laundry duty. He and Ms Desbiens shared in preparation of meals and generally he cleaned up and did the dishes after meals.

[41] Mr. Desbiens testified that his relationship with Ms Desbiens was excellent. He stated that although he was impotent after the 1986 accident, he was still able to be physically intimate with his wife and give her pleasure. He testified that on weekends they would go to the Fieldgate Tavern where western music was played and he would “dance” in the wheelchair.

After living together for 28 years, they married in October 1999. He said that he and Ms Desbiens were planning to take a honeymoon after their marriage but the accident intervened and they never did have a honeymoon.

[42] He was able to take care of his financial affairs. He noted that most banks have ramps and bank machines are wheelchair accessible. He did not have a motorized wheelchair but this did not present any problems for him. There was no limit on how long he could sit in his wheelchair. He attended church located at Erin Mills Parkway and the 401 at least three times a week. He was very active doing community and charitable work in the Knights of Columbus.

[43] He was able to drive a van that did not have a wheelchair lift but was equipped with hand controls and a power slide door opener. He was able to transfer from the wheelchair to the van and pull the folded wheelchair into the van without assistance. He was able to drive long distances without any problems. He and his wife went on vacations together in which he drove. He would drive on visits to his wife's daughter who lives in Riviere du Loup, which is 200 miles east of Quebec City, a 13-hour drive from Toronto.

[44] From 1990 to 1995 his wife worked as a representative for the Shinglers union. Her job required her to visit work sites to check that only union members were being employed. Ms Desbiens did not drive so Mr. Desbiens would drive his wife to the various locations in a van that was provided by the union.

[45] Ms Desbiens testified that when Mr. Desbiens left the Lyndhurst hospital he was able to do most things himself. She was advised not to do everything that he asked her because it may make him too dependent on her. She acknowledged that she did provide some care and assistance to Mr. Desbiens and that she was receiving compensation from the WCB for her services. Three times during any night she would help Mr. Desbiens measure and remove the urine after his catheterization. When he entered the shower, she held his chair and after the shower she would help him dry off. She would apply creams or lotion to his legs. She did not have to help him with dressing or undressing. She helped him with exercises stretching his legs. When he transferred from the bed to the wheelchair all she had to do was hold on to the

wheelchair. Sometimes she had to do extra laundry because he would soil the bed sheets. She would share the cleaning of the bathroom with her husband but she had to clean the bathtub.

[46] Ms Desbiens testified that Mr. Desbiens was very active before the car accident. He liked to go dancing, play basketball, soccer, and go to the bingo and restaurants. They went on holidays and visited relatives. He was able to drive long distances without tiring.

[47] Ms Norlock testified that prior to the car accident Mr. Desbiens functioned so well that she would often forget he was in a wheelchair. He was always busy driving, going to the mall and Knights of Columbus, woodworking, using the computer, doing crosswords, playing basketball and taking the dog for a walk.

The Evidence of Health Care Professionals

[48] When Mr. Desbiens was discharged from the Lyndhurst Hospital on July 17, 1986, the discharge summary prepared by Dr. Geisler noted that, "He's personally independent. He's up and about and active. He can transfer in and out of his automobile. He has been tested with hand controls and can cope well. He requires no help for dressing and undressing."

[49] Dr. Lee expressed the opinion that after Mr. Desbiens became a paraplegic he had developed a high level of independence in his activities of daily living. Dr. Lee stated his opinion of Mr. Desbiens' level of independence was based on his own observations and on the discussions he had with Mr. Desbiens during his visits. He noted that Mr. Desbiens was able to open doors, wheel himself to his office without assistance, was active in the Knights of Columbus and traveled with his wife without difficulty.

Attendant Care before the Car Accident

[50] Mr. Desbiens was entitled to an attendant's allowance from the WCB, which provides funds for hiring attendants to supply in-home care and assistance. The WCB worker, Mr. Greco, attended at Mr. Desbiens' home on February 13, 1987 to prepare an attendant allowance profile report. This report is contained at page 234 of Exhibit 10C. On page 2 of this report he writes:

Mr. Desbiens is fairly active in his manual wheel chair, however he stated that he spends approximately 12-14 hours per day in bed or on the sofa chair. Mr. Desbiens is able to make most of his transfers on his own however he does require some supervision when making transfers to the toilet and bathtub. He also needs assistance in bathing, dressing the bottom portion of his body and with washing his hair.

Mr. Desbiens' bowels must be stimulated by suppository and touch, and his bladder is emptied by catheterizations every 4 hours. He states that he experiences involuntary elimination only when there is lots of movement such as in a moving car or van.

[51] Mr. Greco also noted that Mr. Desbiens required supervision and assistance when bathing, dressing and washing his hair and also required some assistance in bowel and bladder care with some involuntary elimination being experienced causing abnormal linen use. He recommended that a Group 2, Attendance (*sic*) Allowance award be considered and paid retroactively from the date of discharge from the Lyndhurst Hospital. It appears that this recommendation was followed and Attendant's allowance of \$327.76 a month was awarded effective January 22, 1987.

[52] The WCB file indicates that as of July 1, 1989, the evaluation of an injured workers' attendant care needs are to be determined by using an Activities of Daily Living Scale ("ADLS") (Exhibit 10B, page 272). The ADLS provides for 3 levels of attendant care as follows:

Part 1 – Skilled Attendant Care: These are complex health care/hygiene functions which require trained or skilled personnel.

Part 2 – Personnel Attendant Care: Covers routine personnel care, which requires trained personnel.

Part 3 – Basic/Supervisory Attendant Care: Involves basic supervisory functions which require responsible personnel.

[53] The first ADLS that is contained in the WCB file is dated November 1, 1989 (Exhibit 10B page 275). The ADLS form has 6 columns. The first sets out the care requirement such as bowel care or skin care; the second sets out the activity to be performed by the attendant; the third sets out the frequency of such activity; the fourth is the time in minutes required to perform the activity; the fifth is the total minutes daily; and the sixth is the total minutes weekly. The

form concludes with a calculation of monthly attendant's allowance or each level of care, which in Mr. Desbiens' case was as follows:

Care Levels	Hours per month	Rate per hour	Total \$
Skilled care – Part I	34.68	12.00	416.16
SKILLED CARE – PART II	16.97	7.50	127.28
BASIC SUPERVISORY	18.40	5.00	92.00
		GRAND TOTAL	635.44

[54] Where an attendant allowance is paid, entitlement and quality of care is reassessed annually and confirmed, adjusted or discontinued as warranted. The WCB records indicate that as of July 1, 1990 the allowance was increased to \$827.24 due to an increase in the hourly rates and an increase in the amount of time allowed for basic supervisory care.

[55] The most recent ADLS in the WCB file is dated January 1, 1993 and indicates the following breakdown which can be found at page 343 of Exhibit 10B.

CARE LEVELS	HOURS PER MONTH	RATE PER HOUR	TOTAL \$
ATTENDANT/SKILLED	34.71	14.02	486.00
PERSONAL	16.96	8.76	148.56
BASIC SUPERVISORY	48.71	6.35	309.32
		GRAND TOTAL	944.55

[56] Mr. Desbiens was cross-examined at length by Mr. Godden with respect to this ADLS with a view to demonstrating that the WCB had been providing him with an attendant's allowance prior to the car accident for care requirements similar or identical to those which Mr. Desbiens claimed were necessitated by the car accident. Mr. Desbiens explained that he was only claiming the cost of the additional care requirements caused by the car accident. Although in some instances he indicated that he did not know the basis on which the WCB allowance was made. Neither party sought fit to call anyone from the WCB to testify. I accept Mr. Desbiens' evidence in this regard.

The Car Accident

[57] On November 8, 1999 Mr. Desbiens drove with his wife in their 1997 Dodge Caravan to a bank at Keele Street and Major Mackenzie Drive. He was going to the bank to pick up some money for his stepson who was in the roofing business. He parked his vehicle in the parking lot beside the bank and then wheeled himself across the driveway entrance to the parking lot and up to the entrance of the bank. After leaving the bank, while he was wheeling himself across the entrance to the parking lot, the defendants' vehicle struck him on the left side of his wheelchair.

[58] The collision propelled Mr. Desbiens approximately 8-15 feet on to the roadway of Keele Street. He landed on his right side, his right hand hitting the pavement at the same time and ended up on his back. He did not recall hitting his head or losing consciousness. He did recall being helped back into his wheelchair. He felt a lot of pain in his right leg and low back running up to his neck and head with a lot of pressure in his right shoulder. With assistance he was able to get back into the van and drive away from the accident scene. He had heard there was a problem with the ambulance drivers being on strike so he decided to drive himself to the hospital. He drove to his apartment building and picked up Ms Norlock. Although Ms Norlock offered to drive he could not get out of the driver seat because his right leg was so swollen so he drove to the Trillium Hospital.

[59] He was taken into the hospital Emergency department on a stretcher. He says he was feeling bad and signaled to Ms Norlock to come close and told her that he was going to pass out. He then lost consciousness. Ms Norlock left the room to get a nurse. When she returned, the nurse noted that Mr. Desbiens was gray, his mouth was open, and he was non-responsive with no palpable radial pulses. After an I.V. was started Mr. Desbiens awoke and started vomiting. This incident is referred to in the hospital records as a syncopal (fainting) episode. This will be discussed in greater detail later in these reasons.

Treatment and Medical Condition After the Car Accident

[60] X-rays revealed a spiral fracture of the right femur with mild shortening. On November 9, 1999 Mr. Desbiens underwent surgery for an open reduction and internal fixation of the femur fracture. An intramedullary rod with distal locking screw and 2 femoral neck screws were installed. The orthopedic surgeon, Dr. D.F. Wilson, noted that Mr. Desbiens tolerated the procedure well and there were no complications during the operation. While in the hospital it was noted that he had developed a rather large pressure sore on his left buttock with an area about the size of a dollar coin that was open and discharging pus. This was treated conservatively.

[61] Prior to his discharge an occupational therapy assessment was done and his pre-accident performance levels were described as follows: “[Mr. Desbiens] was independent with self-care, transfers, drives adapted van, does groceries with wife, helps with dishes, does light kitchen tasks, wife helps minimally with showers.” I would note that this is largely consistent with Mr. Desbiens’ description of his level of functioning prior to the car accident.

[62] He remained in the hospital until November 19, 1999 when he was discharged from the Trillium Hospital and returned home.

[63] After his discharge Mr. Desbiens first saw Dr. Lee on November 22, 1999. Dr. Lee noted complaints of dizziness, poor memory and sharp pain affecting sleep at night. He noted that Mr. Desbiens’ memory was not as good following the collision as it had been before. Dr. Lee noted that Mr. Desbiens complained of constant severe daily headaches since the motor vehicle accident, mostly in the occipital region. Dr. Lee’s examination of the head and cranial nerves was normal. He arranged for a neurological examination by Dr. Best on November 11, 1999. The results of this exam were unremarkable. To rule out the possibility of an injury to his vertebral artery or a subdural hematoma Dr. Best ordered an MRI of the head, which revealed no abnormalities.

[64] On December 7, 1999, Dr. Lee noted a complaint of blurred vision by Mr. Desbiens. On January 5, 2000 he noted a complaint of pain that was sharp and affecting sleep. On March 2, 2000, Dr. Lee noted that Mr. Desbiens was getting only two to three hours of sleep.

[65] Mr. Desbiens continued to experience persistent headaches and dizziness with neck movements and later he complained of increasing pain and tenderness at his right paracervical muscles. An x-ray of the cervical spine was conducted on March 15, 2000 and reported as normal. On April 7, 2000 Mr. Desbiens had an MRI of the cervical spine that showed no definite cervical cord injury but did reveal a small amount of osteophytes at the C 3-4 level. Dr. Lee prescribed a variety of medications in an attempt to control his headaches but they did not result in any improvement in his pain.

[66] On March 30, 2000, Mr. Desbiens complained of left elbow pain when transferring. He also reported a pressure sore on his buttocks. Dr. Lee noted that there was still scarring at that site when he last examined Mr. Desbiens prior to trial on January 27, 2004 and that Mr. Desbiens had difficulty sitting in one position in his wheelchair for any length of time.

[67] On May 26, 2000, Dr. Lee noted pain at the shoulder and numbness of the fingers. On May 31, 2000 Mr. Desbiens was admitted to the Trillium Heath Centre where Dr. Wilson removed the 3 screws from his right femur but left the intra medullary rod in place.

[68] On June 28, 2000 Dr. Lee noted that according to Mr. Desbiens he was suffering from headaches burning like a fire. On July 20, 2000 he reported burning hip pain.

[69] Because of Mr. Desbiens' persisting problems Dr. Lee referred him to Dr. W. Geisler. Dr. Geisler is a highly regarded expert in the field of spinal cord injury and rehabilitation who had seen Mr. Desbiens on a number of occasions after the 1986 accident and had sent a report to Dr. Lee in January of 1987. On July 30, 2000 Dr. Geisler assessed Mr. Desbiens. In his report to Dr. Lee of August 4, 2000 he notes that Mr. Desbiens told him that he was able to tolerate being in the wheelchair for a few hours and then has to return to bed because he develops too much back pain. Mr. Desbiens stated that the back pain developed after the operation on his leg and described it as a burning and needle pulling type pain which increases as the day goes on and reaches a level of about 9 out of 10 when he transfers from the wheelchair to his bed. Dr. Geisler felt that Mr. Desbiens' pains were soft tissue in origin and that the headaches are mild

myofascial tension headaches. As well, he felt that there was an emerging small component of neuropathic (central cord) pain.

[70] On August 24, 2000 Dr. Wilson reviewed Mr. Desbiens and noted that there hadn't been much change in his symptoms. He continued to have neck pain and back pain and a burning sensation over the outer aspect of his right hip. The swelling over his hip seemed to be improving. Dr. Wilson suspected that it was related to some bleeding from the bone at the site where the screws had been removed.

[71] On September 25, 2000 Dr. Lee noted that Mr. Desbiens complained of left elbow pain. Dr. Lee noted a swelling of the elbow and prescribed a brace and an injection of cortisone.

[72] On October 17, 2000 Mr. Desbiens saw Dr. Geisler again. Dr. Geisler noted that "the pains in his head are unchanged; the pains in his low back, in the low lumbosacral buttock behind the right hip region, were burning in quality and are constant." Dr. Geisler prescribed Neurontin 300 mg. four times daily with increasing dosage. He also thought it would be in order to try a local injection of 2% Xylocaine in an attempt to break the pain cycle.

[73] On January 8, 2001 Mr. Desbiens again saw Dr. Geisler. Dr. Geisler noted that Mr. Desbiens takes up to six Tylenol No. 3 per day, which provides a little pain relief, and one mg. of Ativan at bedtime that permits a little sleep. Dr. Geisler noted that Mr. Desbiens reported frequently awakening during the night because of pain. Dr. Geisler reported that Mr. Desbiens continues to have pain in three major locations: his right knee, behind the right hip region of the lower back and headaches in the right occipital nuchal area and the right supra-scapula area. Mr. Desbiens described pain in his right knee as a "burning pain that sometimes feels like the joint is being pulled apart." This pain fluctuates between 4 and 8 out of 10, being worse when he is doing transfers or when he is in bed trying to straighten his legs. The pain behind the right hip region of the lower back is a pinching and pulling pain. This pain is worse when he moves his legs or when he lies down. Finally, Mr. Desbiens reported that his headache pain fluctuates between a 7 or 8 out of 10, being worse when lying on his side or turning his head. Dr. Geisler

noted that Mr. Desbiens had been taking the Neurontin since November 27, 2000, but provided no relief for his knee and hip pain and was therefore discontinued.

[74] Dr. Geisler suggested that Mr. Desbiens might benefit from a program of deep-water hydrotherapy three times per week for two months. He again suggested a trial injection with Xylocaine or Marcaine into the right occipital nuchal area, right supra-scapula area and the right posterolateral buttock area.

[75] Dr. Lee arranged for Mr. Desbiens to have hydrotherapy at the Credit Valley Hospital. He had 8 sessions but unfortunately it aggravated his pain rather than alleviated it.

[76] In March 2001 Dr. Lee referred Mr. Desbiens to Dr. Mark Greenspoon at the Headache and Neck Pain Medical Centre in Brampton. Dr. Greenspoon's area of specialty is pain management. He generally sees people who have developed chronic pain, as a result of trauma, that has not improved by the use of medications or other treatment modalities.

[77] On March 19, 2001 Dr. Greenspoon assessed Mr. Desbiens. He took a history, examined his medical records and conducted a physical examination of his neurological and musculoskeletal system. He also administered an anxiety-depression questionnaire, which was elevated in both profiles with a score of 16 out of 21. Dr. Greenspoon made the following diagnosis: i) post traumatic right chronic tension type cervicogenic headache, ii) right cervical facet joint irritation, iii) right myofascial cervical-scapular strain, iv) non restorative sleep disorder, controlled with Ativan, v) mood disorder, untreated.

[78] On March 19, 2001 Dr. Greenspoon did some diagnostic nerve blocks in the right occipital, paravertebral and transverse scapular areas using Xylocaine, which is a short acting local anesthetic used for desensitizing nerves. The injection is done by means of a 2 inch long needle inserted about 3/4 of an inch deep all the way to the bone. The injection itself causes a "bee-sting" sensation that many patients find painful until the nerve is desensitized. The purpose of his procedure is to discover whether the patient's pain is emanating from the nerves supplying

the areas injected. This is confirmed if the patient experiences a remission of pain after the injection.

[79] After the diagnostic nerve block Mr. Desbiens experienced complete relief of his right-sided headache, neck and shoulder pain. Within 10 minutes however he did begin to experience a 4 out of 10 left-sided headache but no left-sided neck pain or shoulder pain. This confirmed, in Dr. Greenspoon's opinion, that the C2 through C6 nerves were the source of Mr. Desbiens' pain. He recommended Mr. Desbiens undergo a series of 6-8 nerve block treatments using Xylocaine or Marcaine. Marcaine takes a longer time to act than Xylocaine but the effects last for a longer period of time

[80] The objective of a course of nerve blocks is firstly to see if they would succeed in breaking the cycle of pain. If this occurs then the treatments are discontinued. If they do not achieve this objective then continuation of the treatments is basically palliative to increase the patient's quality of life. In Mr. Desbiens' case the pain cycle was not broken and treatments continued for palliative purposes.

[81] On April 23, 2001 Mr. Desbiens again visited Dr. Greenspoon and was given 18 injections in the head, neck and face. On this occasion Marcaine 5% was used. These nerve block treatments have continued generally on a weekly or twice weekly basis right up until the time of trial. Mr. Desbiens testified that the nerve blocks helped to reduce the pain for 8 to 12 hours after the treatment. According to Dr. Greenspoon's records the duration of the anesthetic effect generally lasted from 12 to 16 hours. The duration of relief and benefit in respect of activities of daily living would vary between 1-6 days.

[82] Dr. Greenspoon acknowledged that most patients are under his care for 2-4 months and on rare occasions as long as 6-8 months. He acknowledged that it was very uncommon to treat a patient for as long as he has treated Mr. Desbiens. However he believed it was justified because of Mr. Desbiens' severe pain and the fact that the nerve blocks were the only thing that kept him functioning. In January 2002 Mr. Desbiens wrote a letter to Dr. Greenspoon requesting

additional weekly treatment “in order to keep my headaches from occurring as frequently as they do.”

[83] Dr. Greenspoon explained to Mr. Desbiens that there were side effects to these types of procedures such as dizziness, post procedure infection, pain around injection site, potential breakdown of tissues from needles and risks such as seizure, and high spinal anesthetic risk, i.e. risk of freezing everything below the injection which would cause the diaphragm to stop and he would have to be sustained on respiratory supports until the nerve of the diaphragm started working again.

[84] Dr. Lee last saw Mr. Desbiens on January 27, 2004. At that point Mr. Desbiens was taking the following medications: Macrobid; Tylenol 3; Metamucil; Ativan; Paxil and Mobicox. Mr. Desbiens continued to complain of pain around the right thigh and further noted that the right knee pain was sometime severe enough to push him out of his chair. Dr. Lee noted that Mr. Desbiens was complaining of daily headaches and that neck pain was still a problem as of the last appointment. He noted that Dr. Greenspoon’s nerve blocks apparently gave him about 18 hours of relief. He said that Mr. Desbiens was still experiencing dizziness to a greater degree than before the car accident. Dr. Lee testified that his reference to low back pain meant L1 through L5 and that the back pain before the car accident seemed to be at a higher level. He stated that Mr. Desbiens’ left and right elbow pain continued and that all other reports of pain after the collision continued from November 1999 to January 2004, when he last saw him prior to the trial.

Level of Functioning After the Car Accident

[85] Mr. Desbiens testified that many of his hobbies and activities have been affected. Since the accident he has only taken one trip to Quebec. Furthermore his lifestyle has changed; he is no longer able to walk the dog; he lies down during the day, spasms take him out of his chair and pain forces him to wake up while sleeping. Mr. Desbiens testified that his wife assists him in transferring to and from bed, assists him with sponge baths twice a day and spends up to one

hour assisting him with bowel movements. Mr. Desbiens testified that he now requires assistance in dressing, changing his clothing and exercising.

[86] Prior to the car accident Mr. Desbiens performed his catheterisation and bowel routine with little assistance. Now his wife must push him on the commode into the bathroom and backwards on to the toilet. He is unable to twist or to bend forward to reach his rectum to perform his digital stimulation and must now rely on his wife to do it. He says that it takes much longer than doing it himself, sometimes as long as an hour.

[87] Since the car accident his wife has to bring the catheter materials into the bedroom once a week and store them in his dresser. She has to bring the materials to him each time he requires them and she sits on the bed and supports his back while he performs the catheterisation. He now requires additional help from his wife for dressing and undressing, personal grooming, cutting his toe nails, putting on and removing the TED stockings, washing his hands, and getting around the apartment and the balcony.

[88] Mr. Desbiens testified that since the car accident he no longer assists with household maintenance including cleaning or vacuuming, and garbage and recycling. Furthermore, he testified that he no longer assists with general household work including making the bed, cleaning the sheepskin sheet, doing laundry, preparing meals, or doing dishes.

[89] Mr. Desbiens testified that after the car accident he stopped driving for a period of time until a new van equipped with a wheelchair lift, electronic, sliding doors and six-way driver seat was purchased in early 2003. He is able to open the doors and lower the lift electronically; his wheelchair is then pushed or wheeled backward on to the lift. The lift takes him up to the seat level where he transfers onto the six-way seat, which moves him in to the driver's position. On good days he can perform the transfer without help but on bad days the right leg gets stuck and he needs help. If the ground is sloping he needs someone to push him on to the lift. He now drives approximately 60% of the time for "short drives" and Ms Norlock drives 40% of the time. Someone else is with him in the car 99.9% of the time.

[90] Since the car accident Mr. Desbiens has resumed some activities such as going to bingo and the casino at Woodbine and playing the slots.

[91] The defendants introduced into evidence videotape of surveillance of Mr. Desbiens that was conducted on August 19, 2000, August 24, 2000 and August 29, 2000. Because the videotape had not been served on the plaintiffs in accordance with the Rules of Civil Procedure, it could only be used for the limited purpose of impeaching Mr. Desbiens credibility. The defendants submitted that the videotape demonstrated that Mr. Desbiens was not being truthful when he described his ability to perform certain functions. I would first point out that in Mr. Godden's cross-examination of Mr. Desbiens he questioned him about his ability to carry on certain functions in August of 2001, which was a year after the surveillance. Even apart from this discrepancy, I did not find any significant inconsistency between Mr. Desbiens' evidence and what was portrayed on the videotape and therefore, it is of no assistance to me in the assessment of Mr. Desbiens' credibility.

[92] Ms Desbiens testified that she helps Mr. Desbiens with activities including dressing and undressing, she assists with his stockings and in the bathroom. She helps him with brushing, foot care, shaving and dentures. She assists him with washing, meals, pushes his wheelchair more and does more laundry. Ms Desbiens also testified that she cannot push him to the bathroom and that Mr. Desbiens needs more help to get out of bed because "he no longer has any muscles".

[93] Ms Desbiens testified that since the car accident Ms Norlock helps clean the apartment whereas prior to the car accident Mr. Desbiens used to do the cleaning. Ms Desbiens testified that she and Ms Norlock now shop for groceries and Mr. Desbiens sometimes assists. She and Ms Norlock split the task of walking and cleaning up after the dog. Ms Norlock does the vacuuming and she and Ms Norlock share the responsibility for doing the dishes. Ms Desbiens assists Mr. Desbiens with his bowel routine and all of his washing and bathing is now done in the bedroom.

[94] Ms Norlock testified that after the 1986 accident Mr. Desbiens needed little help around the apartment and it remained that way until the car accident. After the car accident she essentially stepped into Mr. Desbiens' shoes, doing what he was unable to do because of the injuries that he suffered in the car accident. For quite a while after the car accident he was unable to drive and she did all of the driving. After he got the van with the hoist and the six way seats he was able to get into the van by himself but when the hoist brings him down, he needs help with his legs. She estimated she now does 40% of the driving although she always drives back from the appointments with Dr. Greenspoon because the injections make Mr. Desbiens "dopey". She also helps with the grocery shopping and banking. Prior to the accident Mr. Desbiens was able to gain entrance to the apartment building independently. Now he needs help to get from the parking lot entrance or the main entrance into the apartment building. She spends on average about 30 to 35 hours per week working for the Desbiens.

[95] Dr. Lee opined that, prior to car accident, Mr. Desbiens was independent in the activities of daily living and after the car accident he had lost this independence. He noted that Mr. Desbiens suffered from anxiety and depression and was having trouble sleeping at night because of neck and back pain. He said that while Mr. Desbiens had taken Ativan to assist in sleeping, his sleep was unrestorative. Mr. Desbiens was waking up with tiredness and had low energy level and tired easily. Dr. Lee said that he recommended an orthopaedic power bed for Mr. Desbiens to assist with his complaint of poor sleep. He had not made such a recommendation prior to the car accident.

[96] The plaintiffs called Carol Kelly, an occupational therapist who is a partner in Designable Environments Inc. (DEI), as an expert witness. She has extensive expertise in the assessment of persons suffering from disabilities due to orthopedic, neurological, and spinal cord injury and in determining the needs of injured persons and the costs related to such needs.

[97] Ms Kelly attended upon Mr. Desbiens on May 31, 2001 and June 6, 2001 for a period of approximately 10½ hours. On June 27, 2001 she prepared a lengthy preliminary report outlining the future care requirements for Mr. Desbiens and the associated costs. On January 17, 2003 and

January 7, 2004, Ms Kelly again assessed Mr. Desbiens at his apartment and additional updated reports were prepared.

[98] Ms Kelly testified that she did a “real time” assessment of Ms Desbiens’ care activities by timing them with a stopwatch. She observed such activities as Ms Desbiens’ assistance with transfers, bowel and bladder care, exercise, bed baths, and oral hygiene. In 2003 she noted an increase in the attendant care being provided by Ms Desbiens and noted a further increase in 2004.

[99] In each of her reports and in her testimony Ms Kelly described in great detail her observations and assessment of Mr. Desbiens’ level of functioning in the areas of upper extremity function, lower extremity function, transfers and mobility, activities of daily living, housekeeping and home maintenance. In her report of January 15, 2004 Ms Kelly summarizes her opinion with regard to Mr. Desbiens level of functioning as follows:

In summary, Mr. Desbiens has continued to try to remain active but is finding that his pain and deteriorating posture significantly interfere with his ability to access the community. He states that he had been attending bingo after supper up to five times per week but he now can no longer tolerate these frequent outings and he is becoming more isolated at home. He continues to rely heavily on his wife and family members to assist with his personal care and activities of normal living. Mr. Desbiens’ injuries sustained in the motor vehicle accident of November 8, 1999 have resulted in functional limitations and his quality of life has been significantly and permanently impacted by his pain and physical limitations. His constant and chronic pain, limited ranges of motion at his right shoulder and decreased right upper extremity strength prevent him from performing his essential pre-accident daily tasks independently.

[100] Counsel for the defendants retained Dr. Arthur Ameis, a specialist in physical medicine to conduct a defence medical assessment of Mr. Desbiens. Dr. Ameis has extensive expertise and experience in supervising and carrying out DAC assessments including CAT DAC assessments. Dr. Ameis assessed Mr. Desbiens on November 1, 2001. In the course of his assessment, Dr. Ameis took a biographical and medical history from Mr. Desbiens, conducted a physical examination and reviewed Mr. Desbiens medical and hospital records as well as the reports from the various healthcare professionals who had assessed Mr. Desbiens. He prepared a

detailed medical legal report dated November 1, 2001, and testified at trial. At page 13 of his report, which he adopted in his testimony, Dr. Ameis describes Mr. Desbiens' level of post car accident functioning in this way:

Post –Event Functional Level

According to Mr. Desbiens' narrative he has lost almost all of his previous independence in question that is, he has substantial difficulties with independently supporting himself sitting, or with leaning forwards, so as to dress, undress, bathe, catheterize and perform digital management of his rectal elimination. His wife has become a substantial caregiver and he must also draw upon the services of his granddaughter as a caregiver.

If the 'before and after' reports are all accurate this would represent a very dramatic loss of independence which now appears to be permanent. Its attribution would be chronologically and physiologically, on the basis of the injury sustained in the event of 1999.

Conclusion

[101] The defendants submit that Mr. Desbiens was exaggerating his level of functioning prior to the car accident. However the observations of the health care professionals who assessed him before and after the car accident do not support this submission. Moreover the fact that for more than 13 years before the car accident Mr. Desbiens lived in an apartment that was not modified in any major way to accommodate his paraplegia is compelling evidence of his high level of functioning. I find that prior to the car accident Mr. Desbiens, due to his strong and flexible upper body was functioning at a very high level for a paraplegic and was largely independent in his activities of daily living. I also find that the injuries that he sustained in the car accident caused him to suffer, in the words of Dr. Ameis, a "very dramatic loss of independence" so that he is dependent on others to carry out most of his activities of daily living.

The Legislative Scheme

[102] The relevant provisions of the *Act* and Regulation are as follows:

267.5(3) Despite any other Act and subject to subsections (4) and (6), the owner of an automobile, the occupants of an automobile and any person present at the incident are not liable in an action in Ontario for damages for expenses that have been incurred or will be incurred for health care resulting from bodily injury arising directly or indirectly from the use or operation of the automobile.

(4) Subsection (3) does not apply if the injured person has sustained a catastrophic impairment, as defined in the regulations, arising directly or indirectly from the use or operation of the automobile.

[103] Health care is defined in s. 224(1) of the *Act* as follows:

“health care” includes all goods or services for which payment is provided by the medical, rehabilitation and attendant care benefits provided for in the Statutory Accident Benefits Schedule.

[104] Subsection 5(1) of the Regulation sets out various definitions of catastrophic impairment:

5.(1) For the purposes of subsection 267.5(4) of the Act, “catastrophic impairment” means,

- (a) paraplegia or quadriplegia;
- (b) amputation or other impairment causing the total and permanent loss of use of both arms;
- (c) amputation or other impairment causing the total and permanent loss of use of both an arm and a leg;
- (d) total loss of vision in both eyes;
- (e) brain impairment that, in respect of an accident, results in,
 - (i) a score of 9 or less on the Glasgow Coma Scale, as published in Jennett, B. and Teasdale, G., *Management of Head Injuries*, Contemporary Neurology Series, Volume 20, F.A. Davis Company, Philadelphia, 1981, according to a test administered within a reasonable period of time after the accident by a person trained for that purpose, or
 - (ii) a score of 2 (vegetative) or 3 (severe disability) on the Glasgow Outcome Scale, as published in Jennett, B. and Bond, M., *Assessment of Outcome After Severe Brain Damage*, *Lancet* i:480, 1975, according to a test administered more than six months after the accident by a person trained for that purpose;
- (f) subject to subsections (2) and (3), any impairment or combination of impairments that, in accordance with the American Medical Association’s *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993, results in 55 per cent or more impairment of the whole person;

- or
- (g) subject to subsections (2) and (3), any impairment that, in accordance with the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993, results in a class 4 impairment (marked impairment) or class 5 impairment (extreme impairment) due to mental or behavioural disorder.
 - (2) Clauses (f) and (g) of the definition of "catastrophic impairment" in subsection (1) do not apply in respect of a person unless,
 - (a) the person's health practitioner states in writing that the person's condition has stabilized and is not likely to improve with treatment; or
 - (b) three years have elapsed since the indicent.
 - (3) **For the purpose of clauses (f) and (g) of the definition of "catastrophic impairment" in subsection (1), an impairment that is sustained by a person but is not listed in the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993 shall be deemed to be the impairment that is listed in that document and that is most analogous to the impairment sustained by the person.**

.....

"impairment" means a loss or abnormality of a psychological, physiological or anatomical structure or function.

[emphasis added]

[105] A virtually identical definition of catastrophic impairment appears in the Statutory Accident Benefits Schedule ("SABS").³ In the SABS context, the maximum recoverable amounts for attendant care, medical and rehabilitation expenses and housekeeping expenses are significantly increased for claimants who have sustained a catastrophic impairment.

[106] By virtue of Bill 198,⁴ which applies to accidents occurring on or after October 1, 2003, the "catastrophic impairment" threshold no longer has to be met to permit the recovery of health care expenses in a tort action. Bill 198 instead adopts the "permanent serious" threshold that is now defined in the amended Regulation. However a determination of catastrophic impairment is still required for a claimant to access a higher level of benefits under the SABS.

[107] In this case, the plaintiffs rely on the definitions of catastrophic impairment in clauses (f) and (g) of the Regulation. Both of these clauses require the claimant's impairment to be

³ O. Reg. 403/96.

⁴ S.O. 2003, c. 9.

evaluated in accordance with the *American Medical Association's Guides to the Evaluation of Permanent Impairment*, 4th edition 1993 (the "Guides").

Overview of the Guides

[108] In 1958 the American Medical Association (AMA) struck a committee on the rating of permanent impairments. Over the next thirteen years, that committee and several subcommittees prepared papers on the evaluation of impairments. The individual work product of the committees were published as thirteen separate articles and ultimately collected in 1971 as the first edition of the AMA Guides.⁵ Subsequent revisions led to the second edition in 1984, the third edition in 1990, and the fourth edition in 1993. The Regulation directs the use of the fourth edition which was published in 1993. In March 2002 a fifth edition of the Guides was published. In these reasons, references to the "Guides" are to the fourth edition unless otherwise stated.

[109] The stated purpose of the Guides was to achieve a greater degree of objectivity in estimating the degree of permanent impairments by providing a standard framework and method of analysis. There are various definitions of "impairment" in the Guides, including: "an alteration of an individual's health status" and, "a deviation from normal in the body part or organ system and its functioning". It is defined in the Guides' Glossary as "the loss, loss of use, or derangement of any body part, system, or function." The editors state that the Guides definition of impairment closely parallels that of the World Health Organization (WHO) that has defined impairment as "any loss or abnormality of psychological, physiological or anatomical structure or function."⁶ The definition of impairment in the Regulation is virtually identical to the WHO definition.

[110] Impairments are evaluated in terms of how they affect an individual's "activities of daily living" ("ADL"). These activities include, but are not limited to, self-care, personal

⁵ Lee Samis, "Use and Purpose of the AMA Guides", Practical Strategies for Advocates VII: "Four Tort and Accident Benefits Schemes Ago" The Advocates Society (Ontario) (January 23-24, 1998).

⁶ Guides at 1.

hygiene, communication, physical activity, sensory function, hand function, sexual activity, restful sleep, and social and recreational activities.

[111] The Guides provide the evaluating physician with instructions about: how to acquire relevant information, a format for analyzing recording and reporting the information, protocols or descriptions of ways to evaluate a body part, function or system, and tables that contain impairment percentages. The editors caution however that “an impairment percentage derived by means of the Guides is intended, among other purposes, to represent an informed estimate of the degree to which an individual’s capacity to carry out daily activities has been diminished.”⁷ Physicians are directed to carefully analyze the patient’s history and course of the medical condition and the impact of the condition on the patient’s activities. The physician is directed to utilize the entire gamut of clinical skill and judgment in assessing whether or not the results of measurements or tests are plausible and relate to the impairment being evaluated.⁸ Thus the Guides clearly anticipate that a physician’s judgment and discretion will play an important role in the assessment of a patient’s impairment.

[112] Chapters 2-13 of the Guides each deal with a different organ system. Chapters 14 and 15, deal with mental and behavioral disorders and pain, respectively. Within each organ system chapter, there are specific criteria for assessing different types of impairments. Tables and illustrative figures are provided to assist in translating clinical findings into an impairment percentage. These initial impairment percents are expressed in different ways depending on the part of the body involved but by using the tables provided in the Guides they can be translated into a whole person impairment (“WPI”) percentage. By using a Combined Values Chart⁹ (“CVC”) these percentages can be combined to arrive at an overall WPI percentage.

[113] The CVC uses the formula $A+B(1-A)$ to combine impairments, where A and B are the decimal equivalents of the impairment percentages resulting from the injury. The product of this formula is either the same or less than the number that would be reached if the two impairments

⁷ Guides at 2.

⁸ Guides at 3.

⁹ Guides at 322.

were simply added together. The effect of this formula applies the percentage impairment rating of the second impairment to the remaining functional capacity of the injured person after the first impairment is deducted. For example, if the first impairment results in a WPI of 25% then the remaining functional capacity is 75%. Where the second impairment results in a WPI of 10%, this 10% applies only to the remaining 75%, in effect reducing the second impairment to a WPI of 7.5%. In this example the overall WPI is $0.25 + 0.10(1 - .25)$, or 32.5%.

[114] The Guides are used in many jurisdictions in the USA in the context of workers' compensation schemes. The evaluation criteria and impairment percentages appear to be based on the assumption that the person being assessed is able-bodied. There are no specific instructions about how to deal with a patient who, as a result of a pre-existing impairment, carries out their activities of daily living in a different manner than an able-bodied person.

[115] While the editors acknowledge that the Guides may be used in the litigation process, they caution against using the impairment percentages derived, to make direct financial awards.¹⁰ As Lax J. pointed out in *Snushall v. Fulsang*¹¹ the insurance legislation in Ontario appears to require precisely what the Guides themselves discourage. It has also been pointed out that the Guides are not designed to assess treatment or rehabilitation service requirements. Therefore under Bill 59 we have the anomalous situation that the determination of entitlement to recovery of healthcare expenses in a tort action is governed by a set of guidelines that do not address the need for healthcare or the estimated costs thereof.¹²

Catastrophic Impairment

Position of the Plaintiffs

¹⁰ Guides at 5.

¹¹ [2003] O.J. No. 1493 at para. 59 (S.C.J.).

¹² Ronald Kaplan, "Catastrophic Impairment: A New Legal Concept and Its Operational Criteria" Practical Strategies for Advocates VI: The Advocates Society (Ontario) (October 1996).

[116] The Plaintiffs concede that they bear the burden of proving that Mr. Desbiens falls within the exception set out in s.267.5 (4). They submit that they have satisfied this burden on the following basis:

1. Mr. Desbiens sustained a catastrophic impairment as defined in clause (g), namely... a Class 4 (marked impairment) due to a mental or behavioural disorder.
2. Mr. Desbiens sustained a catastrophic impairment as defined in clause (f) when the impact of these impairments is considered in the context of his pre-existing paraplegia.
3. In the alternative, on a proper interpretation of the Regulation, it is in accordance with the Guides to assign percentage ratings to impairments assessed under Chapter 14 (psychological impairments) and combine them with the impairments assessed under the organ system chapters (physical impairments), for the purpose of determining whether Mr. Desbiens' impairments meet the definition of catastrophic as defined by clause (f).
4. If the interpretation in paragraph 3 is adopted, Mr. Desbiens' impairment meets the definition of catastrophic as defined by clause (f) even without considering his pre-existing paraplegia.
5. If the Regulation cannot be interpreted as set out in 3), then it infringes s. 15 of the *Charter* and a provision permitting this combination of physical and psychological impairments should be read into the legislation.

Did Mr. Desbiens Sustain a Catastrophic Impairment as Defined in Clause (g)?

[117] According to clause (g) an impairment is considered to be catastrophic if, in accordance to the Guides, it results in a class 4 (marked impairment) or class 5 (extreme impairment) due to mental or behavioural disorder. The Guides deal with these disorders at Chapter 14.

[118] Dr. Allan Finlayson, a neuropsychologist, testified as an expert witness for the plaintiffs. A neuropsychologist is a psychologist who specializes in the psychological aspects of brain dysfunction including the diagnosis, treatment and rehabilitation of persons suffering from cognitive deficits and psychological dysfunction. Dr. Finlayson has many years of experience in this area. He is quite familiar with the use of the Guides in the assessment of psychological impairments. He was a member of the Minister's Committee, which developed protocols and guidelines for clinicians doing evaluations under Chapters 4 and 14 of the Guides.

[119] At the request of counsel for the plaintiffs Dr. Finlayson assessed Mr. Desbiens on February 6, 2002. He interviewed both Mr. and Ms Desbiens and conducted a neuropsychological evaluation that involved the administration of a broad range of psychological tests designed to assess Mr. Desbiens' current neurocognitive and psychological functioning.

[120] The attention, concentration and memory tests revealed that Mr. Desbiens had significant difficulties with divided attention and verbal and visual memory. Dr. Finlayson considered Mr. Desbiens' limited education and the fact that the tests were conducted in English were relevant factors in explaining certain low scores on measures of verbal and language function.

[121] The tests of personality and emotional functioning revealed the presence of significant levels of emotional distress. Mr. Desbiens was given the Beck Depression Inventory (BDI) and the Beck Anxiety Inventory (BAI) in which patients are presented with a list of symptoms and asked to endorse any that are present. Mr. Desbiens' scores indicated the presence of severe depression and severe anxiety. On the BDI he endorsed, among other things: profound feelings of punishment and worthlessness, significant losses in pleasure and interest, complete loss of energy and libido, irritability and thoughts of suicide. On the BAI, he endorsed, among other things: fear of the worst happening, an inability to relax, nervousness, feelings of terror (particularly when in front of driveways) and physiological symptoms of numbness and tingling (in his right hand only) and abdominal discomfort.

[122] Mr. Desbiens also completed the Minnesota Multiphasic Personality Inventory – 2 (MMPI-2) which are measures of personality and emotional functioning. His responses suggested the presence of extreme distress with limited personal resources for coping. The most elevated scale suggested a high level of anger, suspicion, and sensitivity. He also endorsed a number of symptoms related to worry, anxiety, tension and depression. Mr. Desbiens also completed the Multidimensional Pain Inventory (MPI) that produced a profile reflective of a dysfunctional coping style.

[123] Based on his assessment in February 2002, Dr. Finlayson concluded that Mr. Desbiens was experiencing significant psychological distress that further compromised his function. He made a DSM-IV diagnosis of “chronic pain disorder with both psychological factors and a general medical condition”. He also suggested the possibility of a DSM-IV diagnosis of “adjustment disorder with mixed anxiety and depressed mood”.

[124] Dr. Finlayson met with Mr. Desbiens again on January 5, 2004. He found that Mr. Desbiens appeared to be more depressed than he was in 2002 and that his pain and related psychological distress were playing an increasing role in his dysfunction. He expressed the opinion that as a result of the accident, Mr. Desbiens met the DSM-IV criteria for an adjustment disorder with depressed mood as well as a chronic pain disorder with psychological factors and a general medical condition. In his opinion these disorders have caused Mr. Desbiens to suffer impairments that fall within Chapter 14 of the Guides. I have no hesitation accepting this opinion. I note that none of the defendants’ experts commented on this particular aspect of Dr. Finlayson’s evidence.

[125] I also find the evidence of Dr. Berry, Dr. Greenspoon, Dr. Lee and Mr. Desbiens supports Dr. Finlayson’s opinion. Dr. Berry a well-known specialist in neurology and psychiatry examined Mr. Desbiens at the request of the defendants. In his report dated January 29, 2003, which was admitted into evidence on consent, he concluded that Mr. Desbiens suffered from a significant emotional disturbance. He stated at page 22:

Symptoms of anxiety, recurring nightmares of being struck by a car, irritability, feelings of depression, fear of being on a sidewalk near a driveway, of approaching automobiles and of going out alone became prominent after the accident as a form of post-traumatic anxiety state and depression attributable to the accident and its circumstances. These post-traumatic emotional symptoms have continued and he has not received any counseling for them. He does receive medication for anxiety and depression. He has also been receiving injections into the head, neck, shoulder and right hip region. He does not appear to be the type of patient who would benefit significantly by counseling and it is probable therefore that a significant level of depression and anxiety will persist as a permanent complication of the accident.

[126] Dr. Greenspoon saw Mr. Desbiens in 2001 and diagnosed what he called an untreated mood disorder that stemmed from the accident. He also administered an anxiety-depression questionnaire. The results were elevated in both profiles.

[127] Dr. Lee diagnosed Mr. Desbiens with reactive depression resulting from the accident and on June 11, 2001 he prescribed the anti-depressant medication Paxil, which Mr. Desbiens is still taking. Although he finds it helpful, his symptoms persist.

[128] After identifying a Chapter 14 disorder, clinicians are directed by the Guides to examine the following four areas of functioning in assessing the severity of the impairment: activities of daily living, social functioning, concentration, and deterioration or decompensation in a work like setting. For each area, five classes or levels of impairment are available: Class 1, no impairment; Class 2, mild impairment; Class 3, moderate impairment; Class 4, marked impairment; Class 5, severe impairment.¹³ It is also possible to arrive at a global impairment classification that takes all four areas of functioning into account.

[129] In the areas of activities of daily living, social functioning, and concentration, Dr. Finlayson found that Mr. Desbiens impairment fell within a Class 3 (moderate impairment) in which the impairment level is described as “compatible with some, but not all, useful functioning.” In the area of adaptation (deterioration or decompensation in a work-like setting), Dr. Finlayson found that Mr. Desbiens’ impairment fell within a Class 4 (marked impairment) in which the impairment levels “significantly impede useful functioning.” It is not disputed that it is sufficient for Mr. Desbiens to establish that his impairment in any one of the areas of functioning meets the requirements of clause (g).

[130] I accept Dr. Finlayson’s findings in the areas of activities of daily living, social functioning, and concentration, persistence and pace. I do not, however, accept his finding that Mr. Desbiens fell within a class 4 (marked impairment) in the area of adaptation (deterioration or decompensation in a work like setting). The focus of the analysis in the area of adaptation is on the psychological stress tolerance of the individual. The Guides explain at page 294:

In the face of such [stressful] circumstances the individual may withdraw from the situation or experience exacerbation of signs and symptoms of a mental disorder; that is, decompensate and have difficulty maintaining activities of daily living, continuing social relationships, and completing tasks. Stresses common to the work environment include attendance, making decisions, scheduling, completing tasks, and interacting with supervisors and peers.

[131] Dr. Finlayson concluded that Mr. Desbiens' activities in assisting his wife with her employment should be considered in this area. Dr. Finlayson stated that Mr. Desbiens was no longer able to carry on these activities for a variety of reasons including: problems with memory, concentration, social interaction, irritability, and inability to get around. Dr. Finlayson stated that "adaptation" includes Mr. Desbiens' ability to get around in his usual manner. He opined that if Mr. Desbiens was unable to get around in his van independently and as frequently as he did before the accident, this amounts to a difficulty adapting in his work-like setting.

[132] I find that the evidence does not support the conclusion that Mr. Desbiens' difficulty with driving stems from an inability to adapt to psychologically stressful situations. Indeed, the evidence appears to demonstrate the contrary. In his report of January 2004 Dr. Finlayson notes that Mr. Desbiens told him that he enjoys driving his van and uses it to "get out" and that "he perceives little fun in his life other than going for a ride." Dr. Finlayson also reported that Mr. Desbiens noted increasing anger while driving, however "he is trying to minimize this." While driving on the highway Mr. Desbiens puts the van on speed control and stays in his lane as a coping technique. I also note that Mr. Desbiens testified that he is still doing about 60% of the driving involving short trips and that Ms Norlock does the other 40%. In my view the evidence shows that any interference with Mr. Desbiens' ability to drive and to get around independently is due to his pain and loss of range of motion and strength rather than a mental disorder that makes him unable to adapt to psychologically stressful situations. Even if his difficulty with driving was due to an inability to tolerate stress, in my view, his level of impairment did not so significantly impede useful functioning in a work like setting so as to amount to a class 4 (marked) impairment.

¹³ Guides Table: Classification of Impairments due to Mental and Behavioural Disorders at 301.

Conclusion

[133] In conclusion I find Mr. Desbiens did not sustain a catastrophic impairment as defined in clause (g).

Did Mr. Desbiens Sustain a Catastrophic Impairment as Defined in Clause (f)?

[134] The plaintiffs' principal expert witness on this issue was Dr. Gail Delaney, a specialist in physical medicine and rehabilitation with extensive experience in the treatment and assessment of spinal cord injury patients and in evaluating such persons' impairments in accordance with the Guides.

[135] Dr. Delaney, at the request of the plaintiffs, conducted two assessments of Mr. Desbiens, one on January 4, 2001, and one on January 10, 2004. She performed a very thorough and precise physical examination of Mr. Desbiens. Since impairment ratings are dependent, in some cases, on the extent of limitation of range of motion she used a goniometer, an instrument designed to precisely measure range of motion. Her findings and opinions following these assessments are contained in her reports of March 25, 2001 and January 11, 2004. On both occasions, she concluded that as a result of the car accident Mr. Desbiens sustained a catastrophic impairment as defined in clause (f).

[136] The defendants' principal witness on this issue was Dr. Arthur Ameis who, at the request of the defendants, conducted an assessment of Mr. Desbiens on November 1, 2001. His report dated November 13, 2001 sets out his findings and opinions. He concluded that Mr. Desbiens did not sustain a catastrophic impairment as defined in clause (f). In his evidence, Dr. Ameis conceded that Dr. Delaney had the advantage, having seen the patient more recently, in terms of being able to comment on the plaintiff's functioning and the permanency of his impairments.

[137] In Dr. Ameis' report dated November 13, 2001 he reviewed Dr. Delaney's March 25, 2001 report. Dr. Ameis' report, as well as his evidence at trial, focused largely on critiquing Dr.

Delaney's use of the Guides rather than the specific clinical findings made by her in the assessment of Mr. Desbiens. Dr. Ameis did not suggest that his clinical findings should be preferred over Dr. Delaney's. Indeed he conceded that Dr. Delaney's examinations were superior to his in that they were more detailed and precise. Dr. Ameis did not take measurements for many of the ranges of motion and other areas covered by Dr. Delaney and he admitted that he "eye-balled" many measurements for which Dr. Delaney had used a goniometer. He suggested however, that the different findings amounted to "inter-observer discrepancy" which undermined their reliability.

[138] Having had the opportunity of hearing Dr. Delaney in chief and a long cross-examination and again in the plaintiffs' reply, I was impressed not only with the depth of her knowledge and expertise in the area under consideration but also with the forthright manner in which she testified. I also find that she conducted careful and thorough examinations of Mr. Desbiens. As noted, Dr. Ameis himself quite candidly acknowledged the superiority of her examinations. I do not agree that the fact that Dr. Ameis made different clinical findings undermined the reliability of Dr. Delaney's findings. Except as specifically noted hereafter, I accept the accuracy and reliability of her clinical findings where they differed from those of Dr. Ameis.

[139] I will now review the experts' findings and application of the Guides to these findings region by region.

Chapter 3 -Musculoskeletal System

a) Cervical Spine

[140] The Guides present two methods for measuring spinal impairments: 1) the Injury Model, also called the Diagnosis-Related Estimates (DRE) model and 2) the Range of Motion (ROM) Model, also called the Functional Model. If the DRE model is used, the clinician determines whether the patient's condition falls within one of the eight categories of cervicthoracic spine impairments set out in the Guides and assigns the impairment percentage that corresponds to that category. If the ROM model is used, the clinician measures the degree

of the patient's flexion, extension, right and left lateral rotation and right and left lateral bending of the neck and assigns the impairment percentage set out in the Guides corresponding to each measurement. The impairment percentage for each of the measurements is then combined in the manner described in the Guides to reach a total impairment percentage for the cervical region.

[141] The Guides direct the clinician to use the DRE model unless the patient's injury does not fit within one of the enumerated categories or more clinical data on the spine is needed to categorize the individual's spine impairment. Dr. Delaney explained that she was more comfortable using the ROM model when she assessed Mr. Desbiens in 2001, but that after her assessment in 2004, she felt that she had enough information to be able to fit him into one of the DRE categories. In 2001, she assigned Mr. Desbiens an impairment of 8% WPI for the cervical region using the Range of Motion model. In 2004, she obtained similar measurements for his active range of motion in the neck and, using the DRE model, fitted him into category II which is described as "minor impairment: clinical signs of neck injury are present without radiculopathy or loss of motion segment integrity."¹⁴ A 5% WPI is assigned to this category. Dr. Ameis agreed with Dr. Delaney's classification of Mr. Desbiens' cervical impairment, however, he had some doubt that the cervical injuries were caused by the accident. Dr. Delaney, on the other hand, was satisfied that the cervical injury was caused by the accident. I accept Dr. Delaney's opinion in this regard.

b) Lumbar Spine

[142] Dr. Delaney did not assign any impairment percentage to the lumbar spine in 2001. She stated that it was difficult for her to obtain accurate range of motion measurements, as she did not have an appropriate bed on which Mr. Desbiens could sit and safely complete the exercise. In 2004, Dr. Delaney had a new bed on which Mr. Desbiens could sit with support, and she measured his lumbosacral range of movement. She stated that he wasn't really able to flex forward or extend backward at all and his right and left lateral flexion ability was also below average.

¹⁴ Guides Table 73 at 110.

[143] Using the DRE model, Dr. Delaney put Mr. Desbiens into category II which is described as “minor impairment: clinical signs of lumbar injury are present without radiculopathy or loss of motion segment integrity.”¹⁵ A 5% impairment of the whole person is assigned to this category. Dr. Ameis did not measure the range or motion for the lumbar spine or make any findings regarding this area.

c) Right Upper Extremity

[144] The right upper extremity was the largest contributor to Dr. Delaney’s rating of Mr. Desbiens’ WPI in both 2001 and 2004. In 2001 she found a 19% WPI due to the right upper extremity. In 2004 Mr. Desbiens’ condition had worsened and she found a 36% WPI.

[145] To arrive at the final WPI percentage for the right upper extremity Dr. Delaney undertook a number of steps: she first determined the impairment of the various parts of the upper extremity. She measured Mr. Desbiens’ active shoulder, elbow, and wrist range of motion as well as his grip strength and the digital sensory loss in his hands. She then translated her findings into impairment percentages. These values are expressed as percentages of UEI and are then combined using the CVC to arrive at a total upper extremity impairment percentage (“UEI”) for the right upper extremity. Finally, the total UEI percentage is translated into a WPI percentage using the table set out at page 20 of the Guides.

[146] For the shoulder, the Guides instruct the clinician to measure flexion, extension, abduction, adduction, internal rotation and external rotation. Her measurements translated into an 11% UEI associated with the right shoulder in 2001 and a 10% UEI in 2004.

[147] For the elbow, the Guides instruct the clinician to measure flexion, extension, pronation and supination. In 2001 Dr. Delaney took these measurements and found no impairment associated with the right elbow. In 2004, her measurements translated into a 2% UEI of the right elbow.

¹⁵ Guides Table 72 at 110.

[148] For the wrist, the Guides instruct the clinician to measure flexion, extension, radial deviation and ulnar deviation. Dr. Delaney obtained these measurements and, using the figures provided in the Guides, translated them into a 3% UEI in 2001. In 2004, Mr. Desbiens wrist range of motion had worsened and the measurements translated into an 8% UEI of the right wrist.

[149] In addition to the range of motion measurements, Dr. Delaney measured Mr. Desbiens' grip strength in his right hand using a Jamar Gripper, a device designed to measure grip strength. In 2001, she obtained three readings of 19kg, 19kg and 20kg. The average of the three readings was 19.3 kg with the normal average for a male in Mr. Desbiens' age group being 45.9 kg. Using the formula provided in the Guides, this translates into a 20% UEI associated with loss of grip strength.

[150] In 2004, Dr. Delaney again measured Mr. Desbiens' grip strength and obtained three readings of: 20kg, 14kg and 11kg. She took the average of these readings - 15 kg – and translated it into a 30% impairment of the right upper extremity. Dr. Ameis took issue with the reliability of Dr. Delaney's grip strength measurements in 2004 because they varied by much more than 20%. He referred to page 64 of the Guides which states “[i]f there is more than 20% variation, one may assume the patient is not exerting full effort.” Both experts agreed that if readings varied by more than 20% that might indicate that the patient is exerting less than full effort. Dr. Delaney took the view that the use of the words “may” rather than “must” in this statement from the Guides, indicates that clinical discretion may be exercised to determine whether there was indeed an issue with the patient exerting less than full effort or whether there was another plausible explanation for the variation in measurements. She stated that she believed that Mr. Desbiens was exerting full effort on the tests and that the reason that his readings decreased dramatically was because of the interference caused by his pain.

[151] I agree with Dr. Delaney's interpretation of this statement from the Guides. My review of the test results in general, and my assessment of Mr. Desbiens as a witness, leads me to the same conclusion as Dr. Delaney. I do, however, have some difficulty in accepting the average of the measurements taken in 2004 as indicative of a level of permanent impairment of grip

strength. Given that the first grip measurement in 2004 was roughly the same as the three measurements in 2001, I am satisfied that an impairment rating of 20% UEI based on the 2001 measurements accurately reflects the impairment associated with loss of grip strength.

[152] In the 2004 assessment, Dr. Delaney found impairment due to sensory loss in Mr. Desbiens' thumb and small finger. The Guides adopt the standard two-point discrimination test to determine digital sensory loss. The examiner uses a testing device with two tips approximately the size of pinheads that are pressed against the patient's finger at various points. The distance between the tips is changed over the course of the test. Thus the examiner can determine the distance between the two tips that is required before the patient senses that he is being pricked by two tips, rather than one.

[153] In 2001, Dr. Delaney's test results revealed that Mr. Desbiens' digital sensory ability was within the normal range except for a very small deviation in the small finger that she did not feel was significant. In 2004, however, the test results for the thumb and the small finger indicated sensory impairments. It is considered normal for a person to be unable to distinguish between one and two pinpoints of contact on a finger when they are 6 mm or less apart from each other. In 2004 Mr. Desbiens could not distinguish two points from one point until they were 7 mm apart on his thumb and 22 mm apart on his little finger. In her report Dr. Delaney translated this into a 6% impairment of the hand resulting from the thumb and a 12% impairment resulting from the small finger.

[154] In her testimony Dr. Delaney acknowledged that the calculations she used in her report to translate these numbers into impairment percentages were in error and she was unable to reconcile the error. Over the course of his testimony, Dr. Ameis prepared a calculation using Dr. Delaney's clinical findings of the sensory loss in the right hand and he arrived at a 2% impairment of the hand resulting from the thumb and a 3% impairment of the hand resulting from the small finger. This translates into an UEI of 5% due to digital sensory impairments. The plaintiffs did not dispute the accuracy of these calculations and I am satisfied that they are correct.

[155] In Dr. Delaney's opinion, the cause of the digital sensory impairments was the injury to the plaintiff's right upper extremity in the motor vehicle accident. She and Dr. Ameis agreed that, generally speaking, the usual physiological rules did not apply to patients with spinal cord injuries. These types of patients, after an injury to a particular part of the body, will often experience numbness in another area. According to Dr. Delaney, the digital sensory impairments were likely caused by the injuries sustained by Mr. Desbiens to his right upper extremity and in particular his shoulder. She was also of the opinion that the loss of sensation could be related to neuropathic pain. I am satisfied that the digital sensory loss described by Dr. Delaney was caused, or contributed to, by injuries sustained by the plaintiff in the motor vehicle accident.

[156] The UEI percentages arising from the accident for Mr. Desbiens' right upper extremity are summarized in the chart below.

Region of Right Upper Extremity	UEI %
Shoulder	10
Elbow	2
Wrist	8
Grip Strength	20
Hand – Digital Sensory Loss	5
Total UEI % using combined values chart	38
Total WPI% using conversion table	23% WPI

d) Left Upper Extremity

[157] Dr. Delaney found range of motion impairments in the left shoulder, elbow and wrist but no grip strength or sensory deficit. She made a diagnosis of lateral epicondylitis, commonly known as "tennis elbow". Dr. Delaney attributed this to the additional strain placed on the left elbow by Mr. Desbiens' using it to compensate for his decreased ability to use his right upper extremity to effect transfers to and from his wheelchair. She felt that the wrist impairment was causally connected to the tennis elbow and that the left shoulder impairment was linked to the impairment of the right shoulder. Dr. Ameis was of the opinion that any impairment to the left upper extremity was not caused by the accident. He noted that Mr. Desbiens did not report his

left upper extremity as having been injured. Although the vehicle did make contact with Mr. Desbiens' left side, there was no suggestion that the impact caused the impairment to the left upper extremity. I accept Dr. Delaney's opinion as to the causal connection between the accident and the impairments in Mr. Desbiens' left upper extremity.

[158] In 2004 Dr. Delaney found a 6% WPI for the left upper extremity. However, Dr. Ameis quite fairly pointed out that Dr. Delaney's clinical findings should have resulted in a WPI of 7% which was comprised of 7% UEI from the shoulder, 2% UEI from the elbow and 2% UEI from the wrist. I accept Dr. Ameis' calculations as correct.

e) Right Lower Extremity

[159] The most serious injury sustained by Mr. Desbiens in the accident was a displaced fracture of his right femur. Both experts agreed that because of his pre-existing paraplegia, the evaluation of the impairment of the right lower extremity posed a particular challenge. Dr. Delaney was of the opinion that Mr. Desbiens' right hip and knee had been impaired as a result of the accident. She acknowledged that there was other impairments to the right lower extremity caused by his pre-existing paraplegia and that these should not be counted in the assessment of the impairment sustained in the accident. She concluded that only certain aspects of the impairment of the right hip and knee were caused by the accident rather than by his pre-existing paraplegia.

[160] As the Guides direct, Dr. Delaney attempted to use the ROM Model for evaluating hip and knee impairment. However, this model is designed to deal with active range of motion. Mr. Desbiens did not have any active motion because of his paraplegia. There is no provision in the Guides for dealing with the evaluation of lower extremity impairments in such a situation. Dr. Delaney opted to use the ROM model with passive range of motion. She measured the passive range of motion of Mr. Desbiens' right and left lower extremities. Acknowledging that there would be some impaired range of motion due to his paraplegia, she took measurements for the left lower extremity, even though it was not injured in the accident, in order to compare the right to the left so as to determine the amount of the impairment attributable to the accident.

Essentially she used the left lower extremity as a benchmark of Mr. Desbiens' pre-accident level of impairment.

[161] Dr. Ameis agreed that the impairment to the right lower extremity had to be captured but criticized Dr. Delaney's approach. In his opinion, it was not appropriate to use a passive range of motion measurements because the tables in the Guides were developed to deal with active range of motion and could not be directly applied to passive range of motion. In his view, because Mr. Desbiens had no active range of motion prior to the accident, he could not properly be given an active range of motion measurement. Further, Dr. Ameis was of the view that the impairment percentages associated with an impaired range of motion were not applicable to a paraplegic because they were premised on the underlying assumption that the loss of range of motion would affect normal motor function and that the person would lose some movement as a result. In Dr. Ameis' opinion "the utter loss of use of both the hip and the knee arose from the paraplegia" so the impairment percentages listed in the tables would not reflect his actual loss of function.

[162] Dr. Delaney conceded that there was no specific direction in the Guides with respect to substituting passive for active range of motion. She emphasized, however, that the Guides were not designed to be applied to paraplegics. Moreover, she opined that an active range of motion and a passive range of motion usually result in the same measurement and that passive range of motion was the best available approximation to active range of motion in this case. She also noted that although there was no precedent for using passive range of motion there was also no precedent for ignoring lower extremity impairment in a paraplegic. Dr. Delaney pointed out that the Guides direct the clinician to consider whether the impairment percentage accurately reflects the loss of function. She concluded that even though the impairment affected his functioning in a different way than it would affect an able-bodied person, the impairment percentage arrived at by using the passive range of motion measurements accurately reflects Mr. Desbiens' loss of function.

[163] In my view the absence of a specific direction in the Guides permitting the use of a passive range of motion does not justify ignoring an obvious impairment in the lower extremity

of a paraplegic simply because he is incapable of being measured for active range of motion. I accept Dr. Delaney's explanation for her decision to substitute passive for active range of motion in the special circumstances of this case. I find that it was a reasonable and informed exercise of her clinical discretion and in accordance with the Guides.

[164] In 2001, Dr. Delaney measured flexion, extension (flexion contracture), abduction, adduction, internal rotation and external rotation of the hip. She also measured flexion and extension (flexion contracture) of the knee. The total combined value that she reached for the right lower extremity was 40% WPI which is the maximum value allowed by the Guides for the lower extremities. She then took the same measurements of the left lower extremity and arrived at a total of 29% WPI. Dr. Delaney presented two alternative methods of accounting for the impairment to the right lower extremity. The first was to include the entire 40% WPI because it, along with the plaintiff's other impairments, brought the total WPI percentage to a number that Dr. Delaney thought accurately reflected his impairment caused by the accident. The second method was to subtract the 29% from the 40% to account for the impairment of the right lower extremity that was likely not caused by the accident. Using the second approach, Dr. Delaney assigned the right lower extremity a WPI of 11%. I find that the second approach more accurately reflects the degree of impairment to the right lower extremity arising from the accident. I will return to discuss the first approach later in these reasons.

[165] In 2004, Dr. Delaney adopted a different approach to the assessment of the lower extremity impairment. She explained that she had become aware of a new approach to assessing hip impairments that was being used in the DACs. This approach was much more conservative because it assigned an impairment percentage only to the worst of the hip range of motion measurements. Dr. Ameis acknowledged that this approach was appropriate and I accept it as the proper approach for the evaluation of Mr. Desbiens' hip impairment. The worst of Mr. Desbiens' measurements in 2004 was hip flexion which resulted in a 4% WPI. Dr. Delaney combined this number with the 4% WPI associated with the right knee to reach a total of 8% WPI. This number was lower than the 11% WPI reached in 2001 because of the more

conservative approach used and because of some minor improvement to Mr. Desbiens' range of motion in his right hip.

Musculoskeletal System Summary

[166] The following chart summarizes the impairment percentages of the Musculoskeletal System:

Region	WPI %
Right Upper Extremity	23
Right Lower Extremity	8
Left Upper Extremity	7
Cervical Spine	5
Lumbar Spine	5
Total	40% WPI

Chapter 4 –The Nervous System

[167] Dr. Delaney first assessed Mr. Desbiens on January 4, 2001. In her report of March 25, 2001 she refers to the syncopal episode in the emergency room and states that the hypotensive episode with resultant hypoxia should be considered as a possible contributor to his on-going memory problems. In this report she did not assign any impairment rating resulting from the hypoxia.

[168] On February 6, 2002, Dr. Finlayson assessed Mr. Desbiens. In his report of July 12, 2002 (at p.15) he notes that the syncopal episode could represent a risk factor for poor outcome following a concussion. However, it is clear from Dr. Finlayson's evidence that he did not feel that Mr. Desbiens had suffered a concussion but rather that his cognitive difficulties resulted from his pain and depression.

[169] On January 10, 2004 Dr. Delaney again assessed Mr. Desbiens. On page 15 of her report of January 11, 2004 she does not include the sleep disorder in her list of Mr. Desbiens' problems. On page 17 she points out that the period of hypotension in the emergency room "could have contributed to his cognitive difficulties". She refers to Dr. Weber's report, (which was not admitted into evidence) in which he assigned a 9% impairment rating for sleep disorders

and she appears to have relied on this opinion to assign a 9% rating for “sleep disturbance”. In the appendix to her report, she includes “sleep arousal disorder” as one of the types of impairments comprising Mr. Desbiens’ WPI, referring to the page number of the Guides containing subsection 4.1e of Chapter 4 and assigns a rating of 9% to this impairment. Table 6 of Chapter 4 describes the impairment criteria for sleep and arousal disorders in 4 different categories and sets out the WPI percentages associated with each

[170] In her testimony, Dr. Delaney justified the inclusion of the sleep disorder as a component of Mr. Desbiens’ WPI on two bases. First, it was her opinion that Mr. Desbiens had probably suffered an injury to the brain as a result of hypoxia (insufficient oxygen) caused by the fainting incident in the emergency department and its sequelae. It was her opinion that this injury to the brain caused a sleep or arousal disorder, which is listed under section 4.1e of Chapter 4 of the Guides. Table 6 of Chapter 4 rates the impairment according to the extent to which the impairment interferes with the daytime alertness of the individual. Dr. Delaney concluded that Mr. Desbiens would fall into the first description under Table 6 namely, “[r]educd daytime alertness with sleep pattern such that patient can carry out most daily activities” which carries with it a WPI rating ranging from 1% to 9%. She felt that the upper limit of 9% WPI was appropriate for Mr. Desbiens.

[171] Her alternative approach was that Mr. Desbiens was suffering from pain that interfered with his sleep and that this was an impairment that could be captured by referring first to Chapter 15, which deals with pain, and then to Chapter 4 dealing with injuries to the nervous system. As I understand it, her reasoning was as follows: Chapter 15 of the Guides permits a physician to quantify the pain-related impairment by looking at the daily activity that is affected by the pain. Based on the history given by Mr. Desbiens, his pain interfered with his sleep. She then determined the Chapter of the Guides that best deals with interference with sleep was Chapter 4 and in particular Chapter 4.1e, Table 6, which sets out the impairment criteria for sleep and arousal disorders

[172] Dr. Delaney also felt that based on the description given by Mr. Desbiens, that the pain that was interfering with his sleep was neuropathic pain, i.e. pain resulting from a dysfunction of

the spinal cord, nerve roots and peripheral nerves. She contended that this placed it within Chapter 4 of the Guides, which deals with permanent impairments resulting from dysfunction of the brain, brain stem, cranial nerves spinal cord, nerve roots, and peripheral nerves.

[173] The defendants submit that the evidence does not support a finding that Mr. Desbiens suffered a brain injury in the car accident.

[174] Mr. Godden points out that in cross-examination Dr. Delaney acknowledged that in order to justify a diagnosis of brain injury two of three criteria have to be satisfied, namely,

- 1) a documented insult to the brain,
- 2) an abnormality on neuropsychological testing to confirm cognitive dysfunction, and
- 3) an abnormality seen on imaging

[175] Dr. Delaney testified that in her opinion the syncopal episode in the emergency room satisfied the requirement of a documented insult to the brain. She acknowledged there was no abnormality on imaging. With respect to the requirement of abnormality on neuropsychological testing, Dr. Delaney acknowledged that there would have to be abnormalities in a cognitive area that is consistent with a reduction in blood supply to the brain. Mr. Godden submits that Dr. Delaney admitted that if there were an opinion from a neuropsychologist indicating that on the balance of probabilities there was no brain damage then this requirement would not be met. I agree that this was the gist of Dr. Delaney's evidence.

[176] Dr. Finlayson was of the opinion that on the balance of probability, there was no brain damage. Thus it is clear that on Dr. Delaney's own evidence the requirement of abnormality on psychological testing has not been met.

[177] Dr. Snow is a neuropsychologist who assessed Mr. Desbiens on behalf of the defendants on November 2nd and 3rd 2002. He conducted a psychological examination and administered a battery of psychometric tests. He prepared a report dated December 30, 2002, in which makes the following statement that he adopted in his testimony at trial:

He did have a brief syncopal episode subsequently, but there was no indication of a change in his behavioural status. He did not have any amnesia for events before the syncopal episode. It is therefore my opinion that Mr. Desbiens did not suffer a brain injury or concussion in the accident. It is also my opinion that he did not suffer any brain injury as a result of his syncopal episode.

[178] It is worthy to note that there was no indication in any of the hospital records that the hospital staff had any concern about brain damage. It would appear that they treated the syncopal episode simply as a fainting spell, started an intravenous and with that intervention Mr. Desbiens awoke and started throwing up. It is also relevant that according to the hospital records and nursing notes, within 15 minutes of his syncopal episode he was awake, alert and oriented.

[179] Dr. Best, a neurologist, assessed Mr. Desbiens on November 11, 1999 and the results of this examination were unremarkable. The MRI of the head ordered by Dr. Best also revealed no abnormalities.

[180] Even apart from the question of causation, there is insufficient evidence to establish that Mr. Desbiens' sleep problems fell within the first category of impairment criteria of Table 6, namely, that he "suffered from reduced daytime alertness with sleep pattern such that patient can carry out most daily activities." While there are several references in the evidence to Mr. Desbiens being unable to sleep because of pain, there is very little to suggest that as a result of the disturbed sleep, he had a consistent problem of reduced daytime alertness that interfered with any of his daily activities.

[181] With respect to Dr. Delaney's alternative approach, I do not accept that Chapter 15 permits the assessor to assign a separate impairment rating that can be taken into account in calculating Mr. Desbiens' WPI. Chapter 15 explains how to evaluate pain. It does not say that pain is a separate impairment for the purposes of arriving at WPI %. The following statement from page 152 of the Guides is instructive: "In some instances pain will affect the proper functioning of a specific organ system. In the Guides, impairment percents shown in the chapters considering the various organ systems make allowance for the pain that may accompany the impairing conditions." In my view this means that in arriving at the percentages assigned to impairment of the musculoskeletal system for example, the Guides have taken into account that

these impairments may be accompanied with pain. Therefore if this pain interferes with Mr. Desbiens' sleep, it has already been included in the impairments found in the musculoskeletal system and cannot be rated as a separate impairment.

[182] I wish to make it clear that I am not making a statement of general application as to the scope of Chapter 15. However, on the evidence before me, I have sufficient doubt about the correctness of Dr. Delaney's alternative approach to justify its rejection.

[183] Even if the pain interfering with Mr. Desbiens' sleep is neuropathic this would not in my view, permit a rating to be made for a sleep or arousal disorder. According to Dr. Delaney neuropathic pain emanates from the spinal cord, nerve roots and peripheral nerves. Table 6 is contained in Chapter 4.1, which only deals dysfunctions of the Cerebrum or Forebrain. Chapter 4.3 deals with disorders of the spinal cord and does not mention sleep or arousal disorders.

Conclusion

[184] Having carefully reviewed all of the evidence on this issue, I have come to the conclusion that the plaintiffs have failed to establish that Mr. Desbiens suffered an injury to the brain or that he suffers from a sleep and arousal disorder as described in Table 6 of Chapter 4. Therefore the inclusion of WPI of 9% on this account is not in accordance with the Guides and cannot be considered in determining whether Mr. Desbiens meets the definition of catastrophic impairment in Clause (f).

Mr. Desbiens' Impairments Superimposed On His Pre-Existing Paraplegia

[185] Without taking into account Mr. Desbiens' pre-existing paraplegia or his psychological impairments, I find that the combination of impairments as set out in the table at paragraph 166 of these reasons resulted in a 40% impairment of the whole person in accordance with the Guides. I will now consider whether Mr. Desbiens sustained a catastrophic impairment as defined by Clause (f), when these impairments are considered in the context of his pre-existing paraplegia.

[186] Both Dr. Delaney and Dr. Ameis agreed that Mr. Desbiens presented a real challenge for a clinician attempting to assess his impairments using the Guides. They both agreed that the Guides are premised on the assumption that one is dealing with a healthy person who sustains impairments as a result of an injurious event. They both were of the view that the Guides permit, and in fact anticipate, that the assessor will exercise discretion where the impairment percentage arrived at does not accurately reflect the real functional impairment sustained by the injured person. They both agreed that in Mr. Desbiens' case the problem was how to assess his impairments in a way that fairly and adequately reflected the impact on his functioning in the context of his pre-existing paraplegia. Each expert approached the solution to the problem in different ways and with different results.

Dr. Delaney's Approach

[187] Dr. Delaney first used what she called a functional approach. In 2001, she arrived at an overall WPI of 60%, which included a right lower extremity impairment rating of 40%. Dr. Delaney acknowledged if she had taken into account the restricted range of motion due to the pre-existing paraplegia, the WPI of the right lower extremity would be reduced to 11% and the overall WPI would be reduced to a WPI of 40%. However she felt that an overall WPI of 60% more accurately reflected the actual functional impairment sustained by Mr. Desbiens in the accident. She opined that a WPI of 40%, when superimposed on Mr. Desbiens' paraplegia, had grave consequences for his ability to function that are not adequately reflected by a WPI of 40%. Viewed in this manner, she was of the opinion that Mr. Desbiens had sustained a catastrophic impairment in the car accident.

[188] In Dr. Delaney's 2004 analysis she initially arrived at a WPI of 55% which included a WPI of 9% for a sleep or arousal disorder but did not take into consideration Mr. Desbiens' pre-existing paraplegia. For the reasons previously stated, I have found this figure should be reduced to a WPI of 40%.

[189] Nonetheless, Dr. Delaney was of the opinion that Mr. Desbiens' impairment must be considered in the context of his pre-existing paraplegia and that when this is done it is clear that he sustained a catastrophic impairment within the meaning of clause (f). She explained that a WPI of 40% to a paraplegic is qualitatively much worse than to an able-bodied person in terms of how it impacts their day-to-day functioning. The difference is best illustrated by considering the impairment to Mr. Desbiens' upper extremities. Paraplegics rely on their upper extremities much more than able-bodied people in their activities of daily living. For an able-bodied person, a serious injury to the arm means that they cannot use the arm for things as lifting, pushing or carrying objects. However, for a paraplegic, an arm injury also affects mobility because it interferes with the ability to operate a wheelchair and to complete transfers in and out of the wheelchair. Because the upper extremity impairment percentages in the Guides were developed with able-bodied people in mind they do not adequately reflect the functional impact of such impairments on a paraplegic.

[190] Dr. Delaney stated that she had no hesitation in finding that the 40% WPI in the context of Mr. Desbiens' paraplegia, easily pushes him over the 55% WPI mark. Dr. Delaney was of the view that in 2001, a WPI of 60% reflected his functional impairment much more accurately than a WPI of 40%. She noted that his overall impairment level was worse in 2004 than in 2001. She conceded that it was very difficult to come up with a precise number to reflect the increased impact of his impairments superimposed on his paraplegia, but, "if pushed", she would go up to about 70 %WPI. She testified that Mr. Desbiens had "almost completely lost the ability to be independent". In her reply evidence, Dr. Delaney stated that even if her WPI rating were reduced to a range of 38% to 50% without regard to his paraplegia, she would still find that Mr. Desbiens had suffered a catastrophic impairment when considered in the context of his pre-existing paraplegia.

[191] Another approach suggested by Dr. Delaney was to look at Mr. Desbiens' impairment percentages relative to his residual capacity after the 1986 accident. She suggested that the impairments suffered by Mr. Desbiens had to be assessed relative to the level of body function he possessed before the car accident rather than against the Guides' standard of the "whole

person impairment”. In her view this approach would offer a more realistic means of assessing the true impact of Mr. Desbiens’ new injuries.

Dr. Ameis Approach

[192] Dr. Ameis testified that if he were evaluating Mr. Desbiens at his CAT DAC, he would first go through the regular routine with the Guides to arrive at an overall WPI percentage. He would then consider whether that figure was plausible in terms of the real impact on the individual’s activities of daily living and if not, he would modify it upward or downward to reflect the real impact. If he chose to increase it, he would set out in writing his reasons for doing so. He felt that the increase would be more defensible if he was able to precisely identify the basis for making the adjustment. In carrying out a CAT DAC assessment he would always use the approach that produced the highest percentage impairment if more than one approach was available.

[193] Dr. Ameis stated that if he was assessing Mr. Desbiens at a CAT DAC he would have arrived at a total impairment rating of approximately 40% WPI after taking into consideration Mr. Desbiens pre-existing paraplegia. He admitted that this was not clearly expressed in any of his reports. In his testimony he detailed the two approaches that he used to arrive at this figure. Dr. Ameis testified that these approaches were intended to specifically account for Mr. Desbiens’ loss of ability to transfer independently.

[194] In the “analogous loss of function” approach, Dr. Ameis took an impairment rating associated with a hernia in the abdominal wall, reasoning that a person with such an injury would experience a decrease in their ability to transfer similar to that of Mr. Desbiens. He assigned a 30% WPI, for this condition. He also included a 2% WPI for dysesthesia resulting from a disturbance of the femoral nerve and a 1% WPI for excessive sweating. He also assigned 1% WPI for an ankylosed (fused) hip – a number that he reduced from the 5% provided for in the Guides because that number applied to a person who was capable of walking prior to the injury. These percentages, combined with the right shoulder WPI of 7% and cervical impairment WPI

of 5% (percentages arrived at by Dr. Delaney which Dr. Ameis accepted) resulted in an overall WPI of 41% that he then rounded down to 40%. Dr. Delaney was of the opinion that using a hernia in the abdominal wall as an analogous loss of function was not appropriate in that this condition was totally unlike the injury that had been sustained by Mr. Desbiens.

[195] In the “activities of daily living equivalent” approach, Dr. Ameis likened Mr. Desbiens’ functioning after the accident to that of a quadriplegic. He found that the DRE Cervicothoracic Category V best captured the difference between a paraplegic and a quadriplegic. A patient in this category has significant upper extremity impairment. The Guides assign a 35% WPI for this condition.¹⁶ He combined this score with the right shoulder impairment of 7% to arrive at an overall WPI of 40%. He did not include the 5% cervical impairment because he felt that this impairment was already included in the 35% WPI. Dr. Delaney felt that Dr. Ameis’ choice of 35% WPI to capture the difference between a paraplegic and a quadriplegic did not take into account the added impact of DRE Cervicothoracic Category V impairment on a person who was already a paraplegic.

Analysis

[196] In my view, the approaches of Dr. Delaney and Dr. Ameis to the unique problem presented by Mr. Desbiens were, on the face of it, similar in their spirit and intent. Both agreed that the Guides provide only estimates of impairment and give the assessor the discretion to increase or decrease an overall impairment percentage if they determine that the percentage does not properly reflect the patient’s actual impairment of function. Both recognized the need to take an approach that would permit an assessor to adequately capture the impact of Mr. Desbiens’ impairments superimposed upon his pre-existing paraplegia. Simply put, their stated objective was the same but their methodology differed.

[197] Dr. Delaney assigned an overall increase in the WPI percentage. Dr. Ameis criticized Dr. Delaney’s methodology on the grounds that she did not provide adequate reasons explaining

¹⁶ Guides at 104.

her increase in Mr. Desbiens' WPI percentage. Thus, he contended, it lacked the "defensibility" of his approach. Reference was made to the following statement at page 64 of the Guides "If the examiner determines that the estimate for the anatomic impairment does not sufficiently reflect the severity of the patient's condition, the examiner may increase the impairment percent, explaining the reason for the increase in writing."

[198] However both experts agreed that once they reached the 55% WPI mark they did not generally spend as much time and detail assessing further impairments. Dr. Ameis was of the view that the further impairments should be listed but not necessarily quantified to the same extent. In considering Dr. Ameis' criticism of Dr. Delaney's approach it is important to remember that Dr. Delaney, in all of her reports, had found the Mr. Desbiens had at least a 55% WPI without taking into account his paraplegia. Furthermore, I find that Dr. Delaney did explain the reason for the increase in writing. On page 27 of her report dated March 25, 2001 she states, "If we keep in mind that he was already paraplegic and we add these new impairments, which are responsible for his alteration in function, we can clearly see that he has sustained a catastrophic impairment in the context of his current state and pre-existing medical condition." As noted, Dr. Delaney elaborated on these reasons in her testimony.

[199] One problem with Dr. Delaney's approach is that if a judge or other tribunal rejects part of the impairment rating, which reduces the WPI to less than 55%, it may be difficult for the judge to determine whether the assessor would still have found the patient to be catastrophically impaired in light of the reduced WPI percentage. Therefore, it would be preferable for assessors to provide a complete analysis and quantification of all impairments; even after a 55% WPI is reached. However, in the special circumstance of this case I find this does not invalidate the methodology employed by Dr. Delaney.

[200] Dr. Delaney testified that on the basis of her 2001 assessment a WPI of 60% or perhaps a little higher, more accurately reflects the impact of Mr. Desbiens' impairment. In 2004 Mr. Desbiens condition worsened and she reached a WPI of 55% WPI without considering the impact of his impairments on his pre-existing paraplegia. She testified that in order to adequately reflect the impact of his impairment, the WPI should be raised to no more than 70%.

In her reply evidence she stated that even if her initial ratings were reduced to the range of 38-51% she would still have found that Mr. Desbiens had sustained a catastrophic impairment when considered in the context of his paraplegia.

[201] With respect to Dr. Delaney's residual capacity approach, I note that she did not go so far as to say that the Guides sanctioned such an approach when assessing a previously impaired person and I was not referred to anything in the Guides that dealt with such an approach. On the record before me it is impossible to decide that such an approach is in accordance with the Guides. However, I must say that there is certain common sense attractiveness to the concept and it certainly does provide a useful perspective on the inadequacy of the Guides in assessing the impairment of persons with pre-existing serious impairments.

[202] I do not accept the approaches used by Dr. Ameis to capture the impact of Mr. Desbiens' impairment on his pre-existing paraplegia. With respect to the analogous loss of function approach that Dr. Ameis adopted at trial it will be necessary to refer in some detail to Dr. Ameis report of November 1, 2001 (2001 report) and Dr. Delaney's report of March 25, 2001 and his testimony at trial to explain how I arrived at this conclusion.

[203] At page 12 of his 2001 report Dr. Ameis deals with Dr. Delaney's impairment ratings of Mr. Desbiens right lower extremity as contained in her report of March 25, 2001. He rejects Dr. Delaney's impairment scores of 30% for the right hip and 14% for the right knee on the grounds that these impairments were not caused by the car accident but rather arose from the paraplegia. He then states, "Therefore, I would not accept that the 33% score suggested by Dr. Delaney is in any way applicable." It is important to note that Dr. Delaney at page 26 of her report had concluded that the combination of the right knee and right hip impairments resulted in a 40% WPI of the right lower extremity. Without considering the right lower extremity, Dr. Delaney concluded that the combined whole person impairment would be 33%. Dr. Delaney testified that she believed that Dr. Ameis' reference to "the 33% score" was an error as it was clear that he was referring to her impairment rating of the right lower extremity of 40% WPI. I must say that my impression on reading Dr. Ameis 2001 report was the same as that of Dr. Delaney.

[204] Following the above quoted statement Dr. Ameis continues:

Final Whole Person Impairment Scoring

Even without entering into a discussion of the applicability of lower extremity scoring of 33%, it should be appreciated that were we to combine 33% with 5% for the neck and 7% for the right upper extremity and 0% for left upper extremity, the AMA Guides table would yield a score of just 41% (which rounds to 40%). This falls well below the 55% threshold of the definition.

[205] In my view this clearly confirms that Dr. Ameis' reference to the 33% score refers to Dr. Delaney's scoring of the right lower extremity impairment. I interpret the above quote as meaning that Dr. Ameis did not accept that there was any causal connection between the right lower extremity impairment and the car accident. However, even if there were no problem with causation, Dr. Delaney's impairment rating for the lower extremity, when combined with Dr. Ameis' ratings for the neck (5%) and right upper extremity (7%) still do not achieve the WPI of 55% required by Clause (f).

[206] When Dr. Ameis was cross examined on this portion of his 2001 report I expected that he would simply say that he had erroneously written 33% instead of 40%. Had he done so, this would not have altered his conclusion that the final WPI fell short of the 55% requirement. However, in his cross-examination Dr. Ameis instead insisted that "the 33% came from an out-of-the-box thought that if Mr. Desbiens' major problem was a loss of ability to transfer then maybe what you should do is try and treat it like someone else who has an inebriated (*sic*) transfer as for example an individual who has a major abdominal wound and loses their abdominal musculature." He then went on to explain how he arrived at the 33% (which has already been described above) and referred to a worksheet which broke the 33% down into its component parts, which he testified was prepared at the time of his report. Dr. Ameis testified that the 33% was intended to capture all of the impairments other than the shoulder and neck. When asked why none of this was mentioned in the report Dr. Ameis testified "I did not explain it in the report and that's my fault...it was not well crafted I did not explain myself."

[207] The plaintiff submitted that Dr. Ameis' explanation of how he arrived at the overall WPI of 40% was essentially an *ex post facto* modification of his initial opinion in order to create a more reasonable position for the defence. They submit that it is clear from Dr. Ameis' 2001 report that he did not accept that there was any impairment caused to the lower extremity by the car accident. The only impairments that Dr. Ameis found to be attributable to the car accident were the 5% rating for the neck and the 7% for the right upper extremity which results in an overall WPI of 12%. Counsel for the plaintiffs point out that this is totally inconsistent with Dr. Ameis' evidence at trial to the effect that had he assessed Mr. Desbiens at a CAT DAC he would have found him to have sustained an overall WPI of about 40%. It seems to me that there is considerable merit to this submission. However, regardless of his motivation I find that his explanation flies in the face of the clear meaning of his report. Dr. Ameis has vast experience in preparing medical legal reports. He acknowledged in cross-examination that great reliance is placed on such reports. I cannot believe that he would have stated a conclusion based on such a complicated and "out of the box" methodology without referring to it in his report. If in fact he did do so, his criticism of Dr Delaney's approach on the grounds that it lacked defensibility, is a case of "the pot calling the kettle black".

[208] I also agree with Dr. Delaney that Dr. Ameis' choice of the hernia to the abdominal wall as causing an analogous loss of function is rather far-fetched.

[209] With respect to Dr. Ameis' activities of daily living equivalent approach I agree with Dr. Delaney that the 35% WPI arrived at by Dr. Ameis does not take into account the impact of this impairment superimposed on Mr. Desbiens' pre-existing paraplegia.

Conclusion

[210] On this issue I prefer Dr. Delaney's opinion to that of Dr. Ameis. I find that when Mr. Desbiens' impairments are considered in the context of his pre-existing paraplegia that he sustained a catastrophic impairment in the car accident as defined in clause (f) of the Regulation.

[211] I am bolstered in my conclusion by the opinion expressed by Dr. Berry, who conducted a defence medical of Mr. Desbiens. In his detailed and comprehensive dated January 29, 2003, Dr. Berry found that Mr. Desbiens had suffered a catastrophic impairment as a result of the car accident. The defendants submitted that Dr. Berry's findings should be discounted, because he made no reference in his report to the Guides. They contend that Dr. Berry must have misapprehended the opinion that was being sought of him. I do not agree. Dr. Berry is qualified both as a neurologist and a psychiatrist. He has vast experience in writing medical legal reports and testifying in Court. Before preparing his report he reviewed the reports and records provided to him by defendants counsel including the reports of Dr. Delaney and Dr. Ameis, which made it clear that the issue was whether Mr. Desbiens had sustained a catastrophic impairment within the meaning of the Regulation. If defendants' counsel, after receiving Dr. Berry's report, felt that he misunderstood the issue, a follow up report should have been requested. In the absence of evidence to the contrary, I find that Dr. Berry when he expressed the opinion that Mr. Desbiens had suffered a catastrophic impairment was using that term as defined in the Regulation.

[212] My conclusion that Mr. Desbiens sustained a catastrophic impairment was reached without regard to any psychological impairment that he may have suffered as a result of the car accident. In the event that I have erred in this finding, I will now deal with the alternative position taken by the plaintiffs.

On a proper interpretation of the Regulation is it in accordance with the Guides to assign percentage ratings to Mr. Desbiens' psychological impairments and combine them with his physical impairments, for the purpose of determining whether his impairments meet the definition of catastrophic as defined by clause (f)?

[213] This question raises the issue of the proper interpretation of the portions of the Regulation which it will be convenient to reproduce here:

S. 5(1)

... ..

(f) **subject to subsections (2) and (3), any impairment or combination of impairments** that, in accordance with the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993, **results in 55 per cent** or more impairment of the whole person;

or

(g) subject to subsections (2) and (3), any impairment that, in accordance with the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993, results in a **class 4 impairment (marked impairment) or class 5 impairment (extreme impairment) due to mental or behavioural disorder**.

.....

(3) For the purpose of clauses (f) and (g) of the definition of "catastrophic impairment" in subsection (1), **an impairment that is sustained by a person but is not listed** in the American Medical Association's *Guides to the Evaluation of Permanent Impairment*, 4th edition, 1993 **shall be deemed to be the impairment that is listed in that document and that is most analogous** to the impairment sustained by the person.

"impairment" means a loss or abnormality of a psychological, physiological or anatomical structure or function.

[Emphasis added]

[214] The defendants submit that since psychological impairments are rated qualitatively (mild, moderate, marked or extreme), rather than by percentages, there is no mechanism in the Guides for combining them with the percentage impairments derived from the organ system chapters to calculate the percentage impairment of the whole person required by clause (f) and therefore it is not "in accordance" with the Guides to do so.

[215] The defendants' submit that only psychological impairments that result in an impairment rating specifically mentioned in clause (g) can be considered under the Regulation. In other words, the defendants submit that psychological impairments that are rated as mild, moderate, marked or extreme are irrelevant in determining whether an individual has sustained a catastrophic impairment as defined by clause (f).

[216] The plaintiffs submit that the Regulation should be interpreted to allow for the consideration under clause (f) of both physical and psychological impairments. The definition of "impairment" in the Regulation includes both psychological, and physiological impairments and since clause (f) refers to "combination of impairments" and not, for example, "combination of physiological impairments," both types of impairments should be included.

[217] The plaintiffs submit that the words "in accordance with" should be interpreted broadly. They submit that, on the facts of this case, the apparent absence in Chapter 14 of percentage ratings for psychological impairments does not preclude their being assigned an appropriate percentage rating. The plaintiffs submit that the evidence of Dr. Finlayson and an examination of the Guides themselves provide an ample evidentiary basis for this conclusion.

[218] Alternatively the plaintiffs rely on subsection 5(3), which provides that impairments that are not listed in the Guides are to be deemed to be the impairment to which they are most analogous. The plaintiffs submit that Dr. Finlayson provided a sufficient evidentiary basis for concluding that Mr. Desbiens' psychological impairments are most analogous to the corresponding emotional or behavioral impairments described in Chapter 4.

[219] The Attorney General is in general agreement with the plaintiffs with respect to the interpretation of the Regulation and says that if this interpretation is adopted, no constitutional question arises. Understandably the Attorney General does not take any position on the evidentiary issue. The Attorney General also submits that the words "not listed" in subsection 5(3) encompasses both impairments that are not identified and impairments that are identified but not assigned any percentage. Since the psychological impairments under Chapter 14 are not assigned percentages they should be considered as "not listed", the Guides permit the assessor to consider the impairment percentage associated with the most analogous impairment.

[220] The defendants argue that "not listed" means "not identified". They contend that since Mr. Desbiens' psychological impairments are identified in Chapter 14 of the Guides subsection 5(3) does not apply.

Analysis

Statutory Interpretation

[221] The modern approach to statutory interpretation has been succinctly described in the following passage from *Driedger on the Construction of Statutes*, 3rd edition¹⁷:

There is only one rule in modern interpretation, namely, courts are obliged to determine the meaning of legislation in its total context, having regard to the purpose of the legislation, the consequences of the proposed interpretations, the presumptions and special rules of interpretation, as well as admissible external aides. In other words the courts must consider and take into account all relevant and admissible indicators of legislative meaning. After taking these into account, the court must then adopt an interpretation that is appropriate. An appropriate interpretation is one that can be justified in terms of (a) its plausibility, that is, its compliance with legislative text; (b) its efficacy, that is, its promotion of the legislative purpose; and (c) its acceptability, that is, the outcome is reasonable and just.

[222] This passage has been cited with approval in a number of decisions that are binding upon me.¹⁸

[223] Legislative purpose should be taken into account in every case and at every stage of interpretation, including the determination of a text's meaning. In so far as the language of the text permits, interpretations that are consistent with or promote legislative purpose should be adopted, while interpretations that defeat or undermine legislative purpose should be avoided.¹⁹

[224] Even in a case where the plain meaning of the words of the provision appear to dictate a particular interpretation this is not determinative; context and purpose are equal guides to statutory interpretation along with the words themselves.²⁰

¹⁷ Ruth Sullivan ed., *Driedger on the Construction of Statutes*, 3rd ed. (Toronto: Butterworths, 1994) at 131.

¹⁸ See *Rizzo v. Rizzo Shoes Ltd.*, [1998] 1 S.C.R. 27 at para 21, *Bapoo v. Co-operators General Insurance Company* (1997), 36 O.R. (3d) 616 (C.A) and *Sutcliffe et al. v. Ontario (Minister of the Environment) et al.* (2003), 65 O.R. (3d) 457 (Div. Ct).

¹⁹ Ruth Sullivan ed., *Sullivan and Driedger on the Construction of Statutes*, 4th ed. (Markham: Butterworths, 2002) at 195.

²⁰ *Rizzo*, *supra*, at para 23.

[225] The presumptions and special rules of interpretation must also be considered. In particular, I must apply the principle that legislation is presumed to have been enacted in conformity with *Charter* norms, in this case, the equality provisions of section 15 of the *Charter*. This would also be consistent with ensuring a reasonable and just outcome.²¹ As noted in *Sullivan and Driedger on the Construction of Statutes*:

The point made by the court in *Zundel*, and in numerous other judgments, is that constitutional documents like the *Charter* set out the norms that are most highly valued in our culture and therefore perform a legitimizing role. For this reason, apart from questions of validity or showing deference to the legislature, it is appropriate for courts to prefer interpretations that tend to promote those principles and norms over interpretations that do not.²²

[226] In addition, Section 10 of the *Interpretation Act*,²³ states:

Every Act shall be deemed to be remedial, whether its immediate purport is to direct the doing of any thing that the Legislature deems to be for the public good or to prevent or punish the doing of any thing that it deems to be contrary to the public good, and shall accordingly receive such fair, large and liberal construction and interpretation as will best ensure the attainment of the object of the Act according to its true intent, meaning and spirit.

[227] The question of whether something is “in accordance” with the Guides also requires an interpretation of the Guides. Non-statutory instruments that have been incorporated into a regulation by reference are considered part of the regulation. In the case of *R v. Collins*²⁴ the B.C. Court of Appeal examined the question of the validity of certain guides that had been incorporated by reference into a regulation but had not been published in the Gazette. Justice Rowles, in response to questions of the validity of the guides stated:

When material is incorporated by reference into a statute or regulation it becomes an integral part of the incorporating instrument as if reproduced therein. In that regard, see *Mainwaring v. Mainwaring*, [1942] 2 D.L.R. 377 (B.C. C.A.) in which McDonald C.J.B.C. referred to the effect of referential legislation in relation to the incorporating statute, at 380:

²¹ See *Canada (A.G.) v. Mossop* (1993), 100 D.L.R. (4th) 658 at 673 (S.C.C.).

²² *Sullivan and Driedger*, *supra*, at 268.

²³ R.S.O. 1990, c. I.11.

²⁴ *R. v. Collins* (2000), 148 C.C.C. (3d) 308 at para. 20 (BCCA).

...Legislation by reference ... has been consistently construed not to be ambulatory in its effect, but to incorporate the extrinsic law as at the date of the Act that is being construed, and to be unaffected by subsequent change of the law incorporated: [citations omitted] The effect of such legislation is as though the extrinsic law referred to was written right into the Act.... [Emphasis added.]

[228] The principle that a referenced instrument is considered part of the Regulation is further supported by the maxim *verba relata hoc maxime operantur per referentiam rit it eis inesse videntur* which means “words to which reference is made in an instrument have the same operation as if they were inserted in the instrument referring to them”. This maxim has been applied in recent caselaw where the court has found that the effect of incorporation by reference is that the material incorporated is considered to be part of the text of the legislation.²⁵

Purpose of the Legislation

[229] The search for legislative purpose can be an extremely difficult task and may require resorting to a number of sources apart from the legislative text, including legislative debates and the history of the legislation.

History of the Legislation

[230] From June 1990 to October 2003, there have been three major legislative regimes governing compensation for persons injured in motor vehicle accidents. The first was the Bill 68 regime, commonly referred to as the Ontario Motorist Protection Plan (OMPP). It came into force on June 22, 1990, with the proclamation of the *Insurance Statute Law Amendment Act*.²⁶ Under that regime, the owner or occupants of an automobile or any person present at the scene of an accident (protected defendants), were not liable either for pecuniary or non-pecuniary damages, unless the claimant sustained a permanent serious disfigurement or a permanent serious impairment of an important bodily function caused by continuing injury which is physical in nature (permanent serious threshold). If the threshold was met, the claimant’s tort

²⁵ *R. v. St. Lawrence Cement Inc.* (2002), 60 O.R. (3d) 712 at para. 18 (C.A.).

²⁶ S.O. 1990, c. 2.

rights were essentially unaffected by the OMPP. Significantly improved benefits were also provided on a no fault basis.

[231] The OMPP was followed by the Bill 164 regime covering accidents occurring between January 1, 1994 and November 1, 1996. Under Bill 164 a claimant who sustained a serious disfigurement or a serious impairment of an important physical mental or psychological function (serious threshold) could recover non-pecuniary damages but protected defendants were not liable for pecuniary damages regardless of the severity of the claimant's impairment. However, there were very generous benefits for income loss, medical, rehabilitation and attendant care available on a no fault basis: for example, attendant care benefits of \$3,000 to \$10,000 a month depending on the nature of the injury, with no overall maximum, and medical rehabilitation to a maximum of \$1, 000,000.

[232] The Bill 59 regime was introduced in 1996 by way of the *Automobile Insurance Rate Stability Act*,²⁷ and the associated regulation, *Court Proceedings for Automobile Accidents that Occur on or after November 1, 1996*.²⁸ Bill 59 applies to accidents that occurred between November 1, 1996 and October 1, 2003. Under Bill 59, claimants are barred from recovering non-pecuniary damages from protected defendants unless the claimants' injuries meet a threshold similar to the Bill 164 threshold except that the permanent requirement was restored. Pecuniary damages are recoverable without meeting any threshold, except for health care expenses, which are not recoverable unless the injured person has sustained a "catastrophic impairment."

Legislative Debates

[233] It is helpful to understand the problem that Bill 59 sought to redress. Under Bill 164 tort claims for pecuniary losses were totally barred. It is clear that one of the main purposes of Bill 59 was to restore the right of innocent victim to sue for pecuniary losses with some limitations. The Honourable Ernie L. Eves Deputy Premier, made the following comment while introducing Bill 59:

With restored tort rights, fairness is brought back into the auto insurance system. People who suffer catastrophic impairments will now be able to sue for health care expenses in excess of the no-fault benefit of \$1 million.²⁹

[234] Mr. Rob Sampson, who moved the second reading of Bill 59 on behalf of the government, shared his allotted time with Mr. Ted Chudleigh, Member for Halton-North. Mr. Chudleigh had these comments as to the purpose of the legislation:

...this legislation restores an innocent victims right to sue for the most important losses over and above benefit losses. Under the current system, there is no access to the courts for economic losses and in some cases at fault motorists are receiving more compensation than those who are not at fault.³⁰

[235] Mr. Flaherty, the Attorney General, speaking for the government of the day, on the second reading of Bill 59 stated:

It got worse in 1994. With Bill 164 the former government, the New Democratic government, did something that had not been done in any modern western society; that is, it took away totally from victims of motor vehicle accidents in the province of Ontario the right to sue for economic loss ...

The new law, Bill 59, which has been introduced and which we debate now at second reading, accomplishes the goal of balance between tort compensation and no-fault compensation. To the extent to which an innocent victim of a motor vehicle accident has a loss of income, that loss of income, to the extent it exceeds the no-fault coverage will be recoverable in tort without the victim having to get over some sort of artificial threshold concerning the nature of the injury. *This is of fundamental importance in terms of fairness to victims of motor vehicle accidents, and reasonable-minded persons looking at this system who understand it realize that this is a fundamental equitable reform that is necessary to put the system back in balance.*³¹ (Emphasis added)

[236] I cannot resist observing the irony that a government that was concerned about the unfairness of an innocent victim having to overcome an “artificial threshold” to recover full loss of income had no such concerns in imposing a catastrophic impairment threshold as a condition

²⁷ S.O. 1996, c. 21.

²⁸ O. Reg. 461/96.

²⁹ Ontario, Legislative Assembly, *Hansard*, (4 June 1996) at 3272 (Hon. Ernie L. Eves).

³⁰ Ontario, Legislative Assembly, *Hansard*, (June 13, 1996) at 3550 (Mr. Ted Chudleigh).

on an innocent victim's right to recover health care expenses. To be charitable, I will assume that the government intended that the catastrophic impairment threshold would be interpreted in a manner that would not deprive an innocent victim of much needed health care expenses.

[237] Indeed, a common thread runs through the remarks quoted above. That is, the intention to restore fairness to the system for the innocent victims of motor vehicle accidents. Thus a major purpose of section 275.5(5) of the *Act* is to ensure that those innocent victims who are in the most need are able to recover health care expenses, perhaps at the expense of those who have less need. The legislature appears to recognize that catastrophically impaired plaintiffs are a special case, and health care costs can be enormous.³² Another important purpose was to control premiums. In my view, however, insofar as health care expenses are concerned, this was to be achieved by the drastic reduction in the level of medical and rehabilitation benefits available on a no-fault basis.

[238] While Bill 59 allows only those who have suffered a catastrophic impairment to recover health care expenses in my view, the text of the Regulation itself indicates that the drafters clearly intended the definition of "catastrophic impairment" to be inclusive rather than restrictive.

[239] Firstly, as has been noted, the definition of "impairment" as meaning "a loss or abnormality of a psychological, physiological or anatomical structure or function" is extremely broad. Indeed it is difficult to conceive of a more inclusive definition.

[240] Secondly, clause (f) ensures that persons who do not suffer any of the specific injuries or conditions described in the other clauses of ss. 5(1), but nevertheless have an impairment, or a combination of impairments, that is so severe that they are among those with the greatest need for health care are able to recover the expenses of that health care. In effect the legislature, with clause (f), included a catch-all provision for the benefit of those who were likely in the greatest need of health care.

³¹ Ontario, Legislative Assembly, *Hansard*, (June 17, 1996) at 3609 (Mr. Jim Flaherty) and see *Chenderovitch v. John Doe*, [2004] O.J. No. 681 (C.A.) at paras. 18-20.

³² *Henderson v. Parker* (1998), 42 O.R. (3d) 462 at 471 (Gen. Div.).

[241] Thirdly, in order to ensure that no impairments were overlooked in determining whether the requirements of clause (f) and (g) were met, the analogous impairment provision, ss. 5(3) was included. This provision comes into play where an impairment is sustained that is not listed in the Guides.

[242] Fourthly, there is nothing in the text of the Regulation that suggests that a combination of physiological and psychological impairments is not permitted. Indeed clause (f) permits *any* combination of impairments, both physical and psychological. The only requirement is that these impairments must result in a 55% WPI "in accordance with" the Guides. While the definition in clause (g) does not include mild or moderate psychological impairments there is nothing in the Regulation that prohibits such impairments from being considered under clause (f). If the intention were to exclude psychological impairments from clause (f), the insertion of the word "physiological" before the word "impairment[s]" would easily have achieved that purpose.

[243] The next question to be determined is whether, on a proper interpretation of the Guides, an assignment of percentages to psychological impairments is in accordance with the Guides. In considering this question it is appropriate to give the words "in accordance with" a fair, large and liberal interpretation so as to ensure the attainment of the object of the *Act* according to its true intent, meaning and spirit.

[244] The second edition (1984) provided ranges of percentages for estimating psychological impairments. They are described in the 4th edition as follows:

Mental functions, such as intelligence, thinking, perception, judgment, affect, and behavior, were considered to fall into five classes, and the ranges were given as follows: normal, 0% to 5%; mild impairment 10% to 20%; moderate impairment, 25% to 50%; moderately severe impairment, 55% to 75%; and severe impairment, more than 75%. Ability to carry out daily activities was estimated as follows: class 1, self-sufficient; class 2, needs minor help; class 3, needs regular help; class 4, needs major help; and class 5, quite helpless. From estimates of the individual's functioning a whole-person impairment estimate could be made.³³

³³ Guides at 301.

[245] Dr. Finlayson testified that while the Guides do not provide ranges of percentages for the psychological impairments set out in Chapter 14, they did recognize that a numeric or ordinal scale might be of use in certain circumstances.³⁴ They also provided general information regarding how a clinician might determine what percentage is appropriate. They further made the decision to include the percentages from the second edition so that they would be available to clinicians for reference. Dr. Finlayson also stated that he did not think that clinicians had stopped using percentage impairments under Chapter 14. He opined that clinicians would use the percentages from the second edition to make their own estimates.

[246] The authors of the Guides note that there are some valid reasons to use ranges of percents for psychological impairments. If this were done, the chapter on mental disorders would be consistent with *Guides* chapters for the other organ systems. Another point is that various systems for estimating disability have developed ranges of percentages; if such estimates were not provided in the *Guides*, the material in the *Guides* on mental disorders might be ignored.

[247] In my view, given the purpose of the legislation as I have found it, these are powerful reasons for supporting the plaintiffs' interpretation.

[248] The argument against providing percentages in the Guides focused on the fact that there are no precise measures of impairment in psychological disorders and the concern that percentages are likely to be used inflexibly by adjudicators, who then are less likely to take into account the many factors that influence psychological impairment. In the end it was this argument that prevailed.

[249] Dr. Finlayson testified that there is a significant amount of overlap between Chapter 4 and Chapter 14. Although Chapter 4 is used when the disturbance or disorder is caused by dysfunction to the brain or central nervous system, the features of the disorder causing the impairment, for example, depression and anxiety, may be common to both chapters. The only essential difference is the cause of the impairment. The Guides state that emotional and behavioural disturbances, such as depression and anxiety disorders, "illustrate the

³⁴ Guides at 301.

interrelationships between the fields of neurology and psychiatry. The disturbances may be the result of neurological impairments but may have psychiatric features as well”.³⁵ In Chapter 4 clinicians are instructed to assign percentages to the impairment classifications within the designated ranges. The Guides state, “the criteria for evaluating these [emotional and behavioural] disturbances relate to the criteria for mental and behavioural impairments.”³⁶ According to Dr. Finlayson, the word “relate” implies some kind of similarity between the two sets of criteria. As can be seen from the chart below, the impairment descriptions under Chapter 4 and Chapter 14 are strikingly similar.

Chapter 4: Emotional and Behavioural Disturbances		Chapter 14: Mental and Behavioural Disorders	
Impairment Description	WPI%	Impairment Description	Class
Mild limitation of daily social and interpersonal functioning	0-14	Mild impairment: Impairment levels are compatible with most useful functioning	2
Moderate limitation of some but not all social and interpersonal daily living functions	15-29	Moderate impairment: Impairment levels are compatible with some, but not all, useful functioning	3
Severe limitation impeding useful action in almost all social and interpersonal functions	30-49	Marked impairment: impairment levels significantly impede useful functioning	4
Severe limitation of all daily functions requiring total dependence on another person	50-70	Extreme impairment: impairment levels preclude useful functioning	5

[250] Dr. Finlayson was of the view that the psychological impairments should be combined with physical impairments to reach an overall percentage of whole person impairment. He stated that if psychological impairments are left out of the WPI% calculations then the individual does not get a fair representation of his whole person impairment. He expressed his disapproval of a situation in which if his patient had a brain injury he could quantify the resulting psychological

³⁵ Guides at 141-2.

³⁶ Guides at 142.

impairment and include it in determining the patient's WPI but if he had no brain injury but had virtually the same psychological impairment there would be no mechanism for doing so.

[251] Although Dr. Berry did not make specific reference to the Guides, he also appears to be of the view that it is appropriate to look at the combination of physical and psychological impairments for the purposes of determining catastrophic impairment. At page 22 of his report he concludes, "[u]nder the circumstances, [Mr. Desbiens appears to qualify] for the catastrophic impairment level arising out of *the combination of the post-traumatic emotional disturbance and the physical limitations* which have now been super-imposed upon his pre-accident paraplegic state." (Emphasis added).

[252] On the basis of Dr. Finlayson's and Dr. Berry's evidence, and on my interpretation of the Guides, I find that it is in accordance with the Guides to assign percentages to Mr. Desbiens' psychological impairments and to combine them with his physical impairments in determining whether he meets the definition of catastrophic impairment under clause (f).

[253] In my opinion this interpretation does not offend the legislative text and it gives effect to the purpose of the legislation. I must now consider whether it produces an outcome that is reasonable and just.

[254] The Defendants relied on *Henderson v. Parker*³⁷ where the court found that an interpretation of s. 267.5(4) which barred a non-catastrophically impaired plaintiff from recovering health care expenses in excess of the benefits in the SABS was not manifestly unjust. At page 473 Heeney J. states:

It may be argued that it is not just to deny the plaintiff the right to sue for health care costs in excess of the SABS maximums. However, the legislation must be looked at as a complete package. Something less than full recovery may be justified in exchange for providing benefits to all parties, regardless of fault. *Similarly, the plaintiff who is less seriously injured may have to take less in order that the limits can be increased for catastrophically injured people who clearly have a greater need.* These competing interests must also be considered within

³⁷ *Henderson, supra.*

the budgetary constraints mandated by a stated desire to hold the line on insurance premiums. Looking at this legislation as an exchange of rights and a balancing of rights, I am not in a position to say that the result is unjust. It is certainly far from perfect, but it is not manifestly unjust. (Emphasis added)

[255] In my view *Henderson* is easily distinguished from the case at bar. *Henderson* dealt with the rights of "a plaintiff who is less seriously injured" rather than one who is catastrophically impaired. Heeney J. found that an interpretation that prevented a person who is less seriously injured from recovering to the same degree as someone who is catastrophically impaired was not manifestly unjust. The issue at bar is whether an interpretation that prevents an innocent victim, whose impairment is partly psychological in origin, from recovering to the same degree as one whose impairment is entirely physical in origin, is just.

[256] Moreover, the exchange of rights or balancing analysis applied by Heeney J. does not support the defendants' interpretation. As I previously observed, in Bill 59 the restoration of the right of innocent persons who are catastrophically impaired to sue for health care expenses was balanced by reducing the health care benefits available under the SABS to those not catastrophically impaired. In my view, there was no balancing involved in determining what constitutes catastrophic impairment under clause (f) or (g). Nor can it be said that if one looks at the complete package, there was a justifiable exchange of rights under Bill 59 so far as the seriously injured plaintiff was concerned. Under Bill 164 all persons in need of health care expenses were entitled, on a no-fault basis, to a maximum of \$1,000,000 in medical rehabilitation benefits and attendant care benefits with no overall maximum. This was reduced in Bill 59 to \$100,000 for medical rehabilitation and \$72,000 for attendant care. Thus Bill 59 gave catastrophically impaired innocent victims the right to sue for health care expenses, which in the vast majority of cases, they could have recovered on a no-fault basis under Bill 164 and also drastically reduced the benefits for the at fault injured person.

[257] In considering the reasonableness of the plaintiffs' interpretation, it is worthy to note that the classification table at page 301 of the Guides makes it clear that psychological impairments can seriously impede virtually every area of a person's functioning. If a mental impairment caused by a dysfunction of the brain or nervous system can be included in the

calculation of whole person impairment, but a similar psychological impairment under Chapter 14 is excluded, then some people with psychological impairments would be denied much needed benefits, not because they were less seriously impaired, but simply because their impairment is attributed to a different cause. This would be an unreasonable outcome. Conversely, the plaintiffs' interpretation produces a reasonable outcome.

[258] In my view, to deprive innocent victims of motor vehicle accidents the right to recover much needed health care expenses because their psychological impairments cannot be combined with their physical impairments in considering their overall WPI is unjust. Moreover, it is inconsistent with the principles and norms of s. 15 of the *Charter* which provides:

15. (1) Every individual is equal before and under the law and has the right to the equal protection and equal benefit of the law without discrimination and, in particular, without discrimination based on race, national or ethnic origin, colour, religion, sex, age or mental or physical disability.

[259] In my view the defendants' interpretation tends to discriminate against persons who have a mental disability. It is trite to state that an outcome that is inconsistent with *Charter* principles and norms cannot be considered just. It should be obvious that I have not engaged a full section 15 analysis and am not commenting on the constitutionality of the Regulation since, on the interpretation that I have accepted it is not necessary to do so.

[260] If I am wrong in concluding that assigning percentages to psychological impairments is in accordance with the Guides, I will now address the alternative submission regarding the proper interpretation of s. 5(3).

[261] The purpose of ss. 5(3) is to ensure that all impairments, as defined in the Regulation, should be included in determining whether the requirements under clauses (f) and (g) have been met. To interpret the word "listed" as only meaning "identified" as urged by the defendants, could defeat this purpose and create a result that is neither reasonable nor just. On the defendants' interpretation, if the Guides identify an impairment ss. 5(3) does not apply. However, where an impairment is identified but a percentage is not provided there is no mechanism for including it in determining whether the person has suffered a catastrophic

impairment as defined in clause (f). Accordingly, I am of the view that it is proper to interpret the words “not listed” as encompassing both impairments that are not identified and impairments that are identified but not assigned any percentage.

[262] The next question is whether the evidence supports the conclusion that the most analogous impairment to Mr. Desbiens psychological impairment is the emotional or behavioural impairments listed in Chapter 4. In my view the evidence of Dr. Finlayson supports an affirmative response to this question. If this is so, there is no doubt that a percentage may be assigned to the impairment and combined with Mr. Desbiens’ physical impairments in determining his overall whole person impairment.

If his psychological impairments are combined with his physical impairments do Mr. Desbiens' impairments meet the definition of catastrophic as defined by clause (f)?

[263] I have already found that Dr. Finlayson’s evidence supports the conclusion that Mr. Desbiens’ psychological impairments, in each of the areas of functioning listed in the Table at page 301 of Chapter 14, fall within the category of moderate impairment. Dr. Finlayson testified that if he were assessing Mr. Desbiens’ impairments under Chapter 4 he would have found them to fall within the description of “moderate limitation of some but not all social and interpersonal daily living functions”³⁸ which permits a WPI percentage ranging from 15%-29%. Dr. Finlayson would have assigned a 25% WPI. Dr. Finlayson testified that the percentages from the 2nd edition can also provide guidance for the appropriate percentage quantification of these impairments and a 25% WPI falls within the percentage range attached to the 'moderate' category in the 2nd edition. The essence of Dr. Finlayson’s evidence in this regard is that the analysis under Chapter 4 and Chapter 14 yields the same result. I therefore find that the appropriate quantification of Mr. Desbiens' psychological impairments is 25% WPI.

[264] If I have erred in accepting Dr. Delaneys’ evidence that Mr. Desbiens’ impairments, when superimposed upon his pre-existing paraplegia, results in an catastrophic impairment as defined in clause (f), but I am correct in my conclusion with respect to the proper interpretation

of the Regulation, then we are left with a 40% WPI associated with the musculoskeletal system and a 25% WPI for psychological impairments, which when combined using the CVC, results in an overall WPI of 55% which satisfies the requirements of clause (f).

Cross-examination of Dr. Ameis On Prior Negative Judicial Comments

[265] In cross examination, plaintiffs' counsel sought to impeach Dr. Ameis' credibility by referring to a number of cases before this court and the Financial Services Commission of Ontario (FSCO) in which negative comments had been made by the judge or arbitrator concerning Dr. Ameis lack of objectivity and impartiality in his role as an expert.

[266] Defendants' counsel objected to this line of questioning on the grounds that the credibility of Dr. Ameis should be determined based on the testimony and demeanor of Dr. Ameis in the case before the court and that the comments made by a judicial officer in another case concerning the nature and quality of Dr. Ameis' evidence is totally irrelevant.

[267] I indicated to counsel at the time that in the interest of expediency I would permit cross-examination on this issue as well as re-examination and that I would rule on the question of admissibility in my reasons for judgment.

[268] While both counsel acknowledged that they knew of cases in which this line of questioning did take place they were unable to provide me with any decision in which the issue was the subject of judicial analysis. However, while the matter was under reserve I came across two cases that, in my view, lead to the conclusion that this line of cross examination is not permissible.

[269] In *R v. Ghorvei*³⁹ the appellant sought to introduce as fresh evidence on appeal, a transcript of a previous trial wherein the main police witness in *Ghorvei* had been the subject of the following comment by the trial judge, "I find that this officer is a compulsive liar. I do not

³⁸ Guides at 142.

³⁹ *R. v. Ghorvei* (1999), 46 O.R. (3d) 63 (C.A.)

believe his evidence at all.”⁴⁰ The Court in *Ghorvei*, after setting out the well-known criteria for the admission of fresh evidence, made the following statement:

The determining issue, therefore, is whether the evidence could reasonably be expected to have affected the result at trial. Of course, it could only have had any effect on the result if it could have been used at trial to impeach Constable Nielsen's credibility.⁴¹

[270] In ruling against allowing this line of cross-examination the court stated:

In my view, it is not proper to cross-examine a witness on the fact that his or her testimony has been rejected or disbelieved in a prior case.⁴²

[271] *Ghorvei* was followed by *R. v. Karaibrahimovic*.⁴³ In this case the Crown, at trial, cross-examined the accused's expert witness on the basis that the expert's testimony was rejected in four other cases. Furthermore Crown counsel specifically cross-examined the expert by putting to him the conclusions of other trial judges apparently critical of his evidence.⁴⁴

[272] Based on the inclusion of this cross-examination *inter alia* the Court allowed the appeal and ordered a new trial. In doing so the Court specifically stated, “there is no legitimate use to which the trier of fact in this case could have put the evidence arising out of [the expert's] cross-examination about four other cases in which it was asserted that his opinion was rejected.”⁴⁵

[273] In my view, I am bound by *Ghorvei* and influenced by *Karaibrahimovic* to hold that the fact that Dr. Ameis has been the subject of negative judicial comment respecting his conduct as an expert witness in other cases is not the proper subject of cross examination in this case.

[274] I do not wish to be understood to say that this line of questioning is impermissible under any circumstances. If a satisfactory evidentiary basis is laid it may become relevant. Plaintiffs' counsel submitted that an adequate evidentiary foundation has been established. He noted that

⁴⁰ *Ghorvei* at para. 24.

⁴¹ *Ghorvei* at para. 28.

⁴² *Ghorvei* at para. 31.

⁴³ *R. v. Karaibrahimovic* (2002), 164 C.C.C. (3d) 431 (Alta. C.A.)

⁴⁴ *Karaibrahimovic* at para. 5.

⁴⁵ *Karaibrahimovic* at para. 12.

Dr. Ameis, in his examination in chief during the qualification process, stated that he testified in court before. On cross-examination Dr. Ameis agreed that he may have testified in court on hundreds of occasion prior to this trial and had given expert evidence in arbitrations on perhaps 50 or 60 occasions. He agreed on cross-examination that an expert medical witness who is not testifying with respect to his or her own patient ought not to act as an advocate and should be as objective or impartial as possible. When asked whether he had testified as an expert on previous occasions he had done so objectively and impartially and not as an advocate, his answer quite fairly was “I’ve tried”. In my opinion this is not a sufficient evidentiary basis to support the introduction of the line of cross-examination sought by the plaintiffs.

Assessment of Damages

Non-pecuniary General Damages

[275] The plaintiffs submit that as a result of the injuries sustained in the car accident Mr. Desbiens’ functioning is similar to that of a quadriplegic. Mr. Wheeler referred to the case of *Bannon et al. v McNeely et al.*⁴⁶ in which Binks J. assessed the damages of the plaintiff in the amount of \$244,600, which at that time was the cap on non-pecuniary damages. Mr. Wheeler submits that a reasonable estimate of Mr. Desbiens’ non-pecuniary general damages is \$230,000 or \$215,000 net of the statutory deductible of \$15,000.

[276] Defendants’ counsel submits that the evidence establishes that Mr. Desbiens was suffering significant amounts of pain prior to the car accident that is not substantially different than the pain he is suffering since the car accident. He points to the numerous references in the notes of Dr. Lee to complaints of headache and neck and back pain for which he was prescribed large amounts of Tylenol 3, a painkiller containing codeine. He submits that Mr. Desbiens’ attempts to explain the location of the pre-accident neck pain and or low back pain as being different from the post-accident neck and low back pain is not reliable or credible.

[277] He notes that Mr. Desbiens is able to drive his new vehicle; he plays bingo and goes to the casino on a regular basis. He submits that the surveillance evidence indicates that Mr.

Desbiens is able to vigorously push his wheelchair along the sidewalk using both his arms in a vigorous fashion.

[278] While the defendants acknowledge that the injuries sustained by Mr. Desbiens have had a significant impact on him, they submit that such impact must be considered in the context of his apparent current activity level. The defendants submit that Mr. Desbiens' general non-pecuniary damages ought to be assessed at a gross figure of \$75,000.

[279] In assessing damages the court must attempt to restore the plaintiff, as far as money can do so, to the position that he would have been in prior to the accident with all of its attendant risks and shortcomings and not to a better position.⁴⁷ Pre-existing medical problems and a predisposition to develop the disability brought about by an accident should be considered in assessing general non-pecuniary damages. This is so because, in assessing the factors that combine to yield an award for general non-pecuniary damages, the jury must compare the individual plaintiff's anticipated lot in life after the accident with that which the plaintiff would have enjoyed but for the accident.⁴⁸

[280] I have set out in some detail the evidence relating to Mr. Desbiens' medical condition and level of functioning before and after the car accident, which does not need to be repeated here. I have found that before the car accident Mr. Desbiens was functioning at a fairly high level for a paraplegic and that due to his injuries in the car accident he suffered a dramatic loss of independence. It must not be forgotten he had been a paraplegic since 1986 and had considerable complaints of pain and discomfort prior to the car accident. Without repeating the testimony of Dr. Lee and the contents of his clinical notes and records that Mr. Desbiens complained of chronic back pain on numerous occasions between the 1986 accident and the car accident. Dr. Lee rated most of the back pain as being moderate in nature but some as quite severe. In assessing Mr. Desbiens' pain levels before the car accident one must have regard for the large

⁴⁶ (1994), 22 O.R. (3d) 396 (Gen. Div.).

⁴⁷ See *Athey v. Leonati*, [1996] 3 S.C.R. 458.

⁴⁸ *Koukounakis v. Stainrod* (1995), 23 O.R. (3d) 299 at para. 14 (C.A.).

quantities of fairly powerful painkillers he was consuming on a regular basis for the 13 years prior to the car accident

[281] *The Bannon* case does not advance the plaintiffs' position. Mrs. Bannon was an unusually active person, in excellent health and enjoying a rewarding and satisfying life prior to the motor vehicle accident. She golfed, played darts, bowled, ice-skated, danced, traveled, and looked after her husband. She went to nursing home benefits and retirement homes, sang in the church choir and was a member of the women's group in the church and volunteered at the local hospital. She sustained a spinal chord injury in the accident which rendered her an incomplete quadriplegic with some use of her limbs but was largely dependent on others for all of her activities of daily living. She suffered from chronic intense burning pain all over her body, urinary incontinence and the court found that she had been deprived of a superior quality of life and was in constant pain with no hope of improvement. Mrs. Brannon's pre-accident condition, and what she might have anticipated had the accident not occurred, was far more favourable than that of Mr. Desbiens.

[282] On the other hand, the fact that Mr. Desbiens has subjected himself to the pain and inconvenience of regular nerve block treatments, described by Dr. Greenspoon, for the sake of temporary pain relief strongly supports the conclusion that his pain levels sharply increased after the car accident.

[283] I realize that comparative analysis of other cases generally offers limited assistance. The assessment of general non-pecuniary damages ultimately depends on a mix of factors, many of which are peculiar to a particular plaintiff. An assessment of an amount close to the Trilogy cap in this case would ignore the realities of Mr. Desbiens' condition in life prior to the car accident.

[284] I assess the non-pecuniary general damages of Mr. Desbiens at \$165,000 from which there must be deducted the statutory deductible of \$15,000 resulting in a net assessment of \$135,000 under this head of damages.

Family Law Act⁴⁹ Damages of Ms Desbiens

Section 61(2)(e)- Care Guidance and Companionship

[285] The plaintiff submits that Ms Desbiens suffered a series of dramatic losses due to the injuries sustained by Mr. Desbiens. He testified that the increasing strain of his condition as a result of the accident as she grew older was causing her to become more and more demanding and angry as the situation was becoming “unbearable”. Mr. Wheeler made reference to the evidence of Mr. Desbiens in which he noted that his wife had raised the possibility of leaving him at some point.

[286] Ms Desbiens, in her evidence, noted that her relationship with Mr. Desbiens was not as good as it was before the collision and that they are no longer physically intimate. They participate in fewer activities together. She had mentioned her problems to her family physician who has attempted to find a French-speaking counselor but has thus far been unsuccessful. Mr. Norlock noted that Ms Desbiens is tired, depressed and angry, that Mr. Desbiens is being more demanding, and that they argue. Plaintiffs’ counsel referred to the case of *Peddle (Litigation guardian of) v. Ontario (Minister of Transportation)*⁵⁰ in which the spouse was awarded damages for loss of guidance, care and companionship in the amount of \$80,000. The plaintiff submits that Ms Desbiens’ award should also be at the high end and that a gross award of \$80,000 minus the statutory deductible of \$7,500 would appropriately reflect Ms Desbiens’ loss.

[287] The defendants point out that Ms Desbiens is 10 years older than her husband and it would be expected that there would be some deterioration of their relationship in any event due to his paraplegic state. They submit that she continues to interrelate with him in her ongoing daily activities much the same as she did prior to the car accident. They submit that her damages should be assessed at a gross figure of \$15,000.

⁴⁹ *Family Law Act*, R.S.O. 1990, c. F.3 (“FLA”).

⁵⁰ [1997] O.J. No. 1874 (Gen Div.).

[288] In my view, Ms Desbiens has suffered a significant loss of care and companionship due to the injuries sustained by Mr. Desbiens in the car accident. Not surprisingly I find the defendants' figure too low and the plaintiffs' figure too high. In all the circumstances I find that an appropriate assessment of damages under section 61(2)(e) of the *FLA* is \$40,000, which after applying the \$7,500 deductible results in a net assessment of \$32,500.

Section 61(2)(d) - Reasonable Allowance for the Value of Nursing, Housekeeping or Other Services

[289] The plaintiffs submit that Ms Desbiens is entitled to an award under section 61(2)(d) of the *FLA*. This subsection provides that the dependant's claim for damages may include a reasonable allowance for the loss of income or the value of nursing, housekeeping or other services provided for the injured person, or for the loss of income incurred by the claimant as a result of providing such services. The plaintiffs submit that an award of this type can be made even if the services fall within the definition of healthcare regardless of whether Mr. Desbiens is found to have sustained a catastrophic impairment.

[290] The defendants submit that unless the injured plaintiff sustains a catastrophic impairment a family law claimant cannot recover under section 61(2)(d) for any services that would fall within the definition of health care expenses. They submit that the opening words of the section 267.5(3) of the *Act* which states, "Despite any other Act..." indicates an intention on the part of the legislature to exempt the protected defendants from a claim for damages for health care expenses regardless of the statutory basis for the claim.

[291] In view of my finding on the catastrophic impairment issue it is not necessary for me to determine this question. However in the event I am found to have erred in my finding and since the matter was fully argued, I feel it is appropriate for me to express my views on this question.

[292] The plaintiff submits that a proper interpretation of section 267.5 (3) of the *Act* leads to the conclusion that the legislature did not intend to exclude a claim under section 61(2)(d) of the

FLA. They submit that if that had been the intention of the legislation they would have included language similar to that which was included in section 267.5 (5), which reads:

Despite any other Act and subject to subsection (6), the owner of an automobile, the occupants of an automobile and any person present at the incident are not liable in an action in Ontario for damages for non-pecuniary loss, including damages for non-pecuniary loss under clause 61(2)(e) of the *Family Law Act*...(Emphasis added)

[293] Thus the plaintiffs contend that the failure of the legislature to specifically mention s. 61(2)(d) of the *FLA* in section 267.5(3) of the *Act* evidences an intention that the protected defendants should not be exempted from a claim under section 61(2)(d) of the *FLA* for health care expenses.

[294] I do not accept this interpretation. The language of section 267.5(3) is clear and unambiguous. It protects the named defendants from damages for expenses for health care resulting from bodily injury directly or non-directly from the use or operation of an automobile, regardless of the statute under which such a claim is made.

[295] The inclusion of the reference to section 61(2)(e) in s. 267(5) can be explained by examining Bill 164, the legislation that preceded Bill 59. Pursuant to Section 267.1(1) of the *Act* under the Bill 164 regime, protected defendants were not liable for loss or damage from bodily injury or death arising from the use or operation of a motor vehicle. However section 267.1(2) of that *Act* exempted damages for non-pecuniary loss from the operation of ss. (1) if the serious threshold was met. The pertinent part of section 267(2) reads as follows:

“Subsection (1) does not relieve a person from liability for damages for non pecuniary loss, *including damages for non-pecuniary loss under clause 61(2)(e) of the Family Law Act ...*”. (Emphasis added).

[296] In my view, the italicized words were included so as to ensure that a claim under clause 61(2)(e) of the *FLA* for guidance, care and companionship would be exempted from the operation of section 267.1(1) if the “serious” threshold were met. The drafters quite wisely recognized that in the absence of specific language to the contrary, a claim for loss of care,

guidance and companionship might be interpreted as a pecuniary rather than non-pecuniary loss because of the wording of section 61(1) of the *FLA*, which provides that the named family members “...are entitled to recover their *pecuniary loss* ...” As previously noted, under Bill 59 protected defendants are immune from claims for damages for non-pecuniary loss unless the “permanent and serious” threshold is met. The reference to section 61(2)(e) in section 267.5(5) was carried over from the previous legislation to ensure that immunity to the protected defendants would apply to claims for loss of care, guidance and companionship under the *FLA*.

[297] Ms Desbiens kept no diary or other record of the time that she spent rendering nursing, housekeeping or other services to her husband. Indeed no attempt was made by Mr. Desbiens or his wife in their testimony to quantify the amount of time spent by her on these services. I have described in some detail the type of services that she provided to Mr. Desbiens after the car accident.

[298] Plaintiffs' counsel acknowledges the difficulty in quantifying the amount of time spent by Mrs. Desbiens in rendering the services to her husband. He submits, however, that the best estimate of the type, amount, and value of these services can be derived from the evidence of the Ms Kelly. Ms Kelly testified that she did a real time assessment of Ms Desbiens activities on the occasion of her visits to the Desbiens, home in 2001. She specifically observed activities such as transfers to and from the wheelchair, assistance with urinary and bowel care, stretching, and bed bath and oral hygiene. It is submitted that I should base my award on the basis of Ms Kelly's evidence of the time that it took Ms Desbiens to perform the various services she observed which would amount to approximately 39 hours per week. Ms Kelly testified that the market rate for a personal service worker (PSW) to perform these types of services was \$20.02 per hour in 2004. The plaintiffs therefore submit that an award of \$171,749 should be made as a reasonable allowance to Ms Desbiens for the value of such services.

[299] I accept that Ms Desbiens did provide additional care to her husband as a result of the car accident. However I have considerable difficulty in arriving at the reasonable value of such services. What Ms Kelly observed when she visited the home does not, in my view, provide a satisfactory evidentiary basis on which to make an estimate of the time actually spent by Ms

Desbiens from the time of the car accident to the date of trial. I also do not accept that \$20.02 per hour would be a reasonable hourly rate of compensation during the period in question. If I accepted the plaintiffs' submission, the award to Ms Desbiens, for approximately 4 years of her services, would exceed the award of non-pecuniary damages to Mr. Desbiens.

[300] In *Dube (Litigation Guarding of) v. Penlon Ltd.*⁵¹ Zuber J. noted that the estimate of these types of losses was more art than science. I agree. Doing the best that I can on the evidence before me I assess the value of her damages under section 62.1(d) at \$45,000 of which I would allocate \$35,000 as being within the definition of health care expenses and the balance falling under the category of housekeeping and other services.

Special Damages

Housekeeping

[301] The plaintiff claims the sum of \$77,000 as an out-of-pocket expense, being the amount alleged to have been paid to Ms Norlock, for housekeeping services at the rate of \$350 per week to the date of trial. The plaintiffs claim Mr. Desbiens would have performed these services had the car accident not happened.

[302] Mr. Desbiens testified that Ms Norlock was hired to help Ms Desbiens and also to replace the work that he was doing in the house after the car accident. When asked how they arrived at \$350 a week, his response was that that was all he could afford to pay her. In her examination-in-chief Ms Norlock testified that the \$350 figure was arrived at so as to ensure that she wasn't losing any income because she was making \$350 per week, on average, from daycare. On cross-examination she admitted that she had never made more than \$230 per week from daycare and that after the car accident she continued to look after one or two children and made between \$50 and \$80 per week. She testified that she averaged about 30 to 35 hours per week doing housekeeping for the Desbiens' and she was originally paid \$10 per hour, which was subsequently increased to \$15 per hour in January 2004. All payments were made in cash. Ms

⁵¹ (1994), 21 C.C.L.T. (2d) 268 at para. 193 (Ont. Gen. Div.).

Norlock did not disclose the income in her tax returns. No claim under the SABS was made for housekeeping expenses paid to Ms Norlock.

[303] Ms Kelly, in her future care report of June 27, 2001, set out in some detail the types of housekeeping activities which she was advised had been performed by Mr. Desbiens before the car accident but not after. These activities involved meal preparation, bed making, laundry, floor care, bathroom cleaning, grocery shopping, pet care and community outings. She prepared a calculation, breaking down into minutes per week the time spent by Ms Norlock on each of these activities and concluded that Ms Norlock was spending 32 hours per week on these activities. In her report of March 5, 2003, Ms Kelly estimated that a reasonable hourly rate for someone to perform these housekeeping services would be \$12 per hour resulting in a weekly expense of \$384 per week or \$19,968 per year. The plaintiffs contend that Ms Kelly's evidence supports the reasonableness of the out-of-pocket claim made for Ms Norlock's services, which was based on an annual amount of \$18,200. Ms Norlock agreed in cross-examination that she was earning about \$22,800 per year in total from the Desbiens, which was much more than she ever made from daycare work.

[304] I find it difficult to accept the plaintiffs' submission. Even though I have found that Mr. Desbiens was functioning at a high level for a paraplegic prior to the car accident, it must be remembered that his paraplegia required him to devote considerable time to his own personal and medical care. As well, he testified that he was involved in a number of social, athletic and community activities. I believe the evidence was, prior to the car accident, Mr. Desbiens would spend approximately 12-14 hours of the day either resting or sleeping. The residence in which Mr. Desbiens was living was approximately 720 square feet with one bathroom. It is reasonable to assume that if it took Ms Norlock 32 hours per week to perform these services, it would take Mr. Desbiens considerably longer. The plaintiff is also claiming the amount of \$5,095 for payments made to Ms Norlock to accompany him as an aide when he went to medical appointments, church, banking and grocery shopping. These amounts total \$5,645 for the period from June 2002 to the date of trial or approximately \$260 per month.

[305] In my view, a more reasonable estimate of the time that it would take to replace the housekeeping services performed by Mr. Desbiens would be 18 hours per week. I accept the hourly rate suggested by Ms Kelly of \$12 per hour. This results in an annual amount of \$11,232. I calculate the period of time from Mr. Desbiens' release from the hospital to the commencement of trial at 4.35 years. I therefore allow the claim for housekeeping services to the date of trial in the amount of \$48,859.20. In my view, housekeeping does not fall within the definition of healthcare expenses and would be recoverable even in the event that Mr. Desbiens did not meet the catastrophic impairment threshold. In that respect I prefer to follow the reasoning set out in *Briggs v. Maybee et al*⁵² and *Lodge v. Regier*.⁵³

Parking

[306] I allow the amount claimed of \$280.00.

Medication

[307] I allow the amount claimed of \$2,568.45 for medications.

Dental

[308] Mr. Desbiens claims the sum of \$1,700 for replacing ill-fitting dentures including a liner. The evidence indicates that Mr. Desbiens' dentures had not been replaced for many years. There is no evidence that the injuries sustained in the car accident are causally connected with the replacement of the dentures or any other dental work. I therefore disallow this claim.

Mileage

[309] Mr. Desbiens claims the sum of \$2,409.24 for mileage representing trips to doctors and other health care professionals. Mileage expense incurred for this purpose is a benefit which is payable under the SABS as part of medical rehabilitation, however, it is subject to a 50 kilometer

⁵² (2001), 53 O.R. (3d) 368 (S.C.J.).

⁵³ [2002] O.J. No. 5517 (S.C.J.).

deductible for each round trip. All of the mileage claimed is less than 50 km roundtrip. The defendants do not dispute the amount of the claim but submits that it is only recoverable if a finding of catastrophic impairment is made. I therefore allow the claim in full.

Snow Removal

[310] Mr. Desbiens claims the sum of \$250 paid for snow removal around the outdoor parking lot so as to permit him to gain entrance to his car. I allow this amount in full.

Aide to Medical Appointments, Church, and Banking/Groceries

[311] Mr. Desbiens claims a total of \$5,645.00 being the amount paid to Ms Norlock from the date of Mr. Desbiens' settlement of the SABS claim (June 5, 2002) until the date of trial. The SABS provide a benefit for transportation for an aide or attendant to and from treatment sessions as part of medical rehabilitation and attendant care benefits. The defendants submit that this claim is not recoverable unless catastrophic impairment is found. They also question the amount claimed since all of these payments were allegedly made to Ms Norlock in cash with no receipts being given. In my view the amount of \$5,645.00 is excessive for reasons that I have referred to under the heading of Housekeeping. I therefore allow this claim in the amount of \$2,300.00.

Vehicle Modifications

[312] Mr. Desbiens claims the sum of \$34,849.60 as the costs of the modification of a 2003 Ford Econoline van that he purchased from Sparrowhawk Industries on August 26, 2002. The purchase agreement (the "PA") indicates that the price of the van before any modifications was \$34,074 and the price of the modifications was \$34,582. Both prices are before PST and GST. The total price of the vehicle after conversions was \$68,656 plus PST of \$4,341.12 and GST of \$2,541.28 for a total of \$75,538.60. The PA indicates that Mr. Desbiens paid a deposit in the amount of \$34,849.60 and the balance of the purchase price of \$40,688.40 was funded by the WSIB. It is clear that neither GST nor PST was charged on the total price and since there is no breakdown of the tax between the van price of the van and the modifications it is impossible to determine what the cost of the each was inclusive of tax.

[313] Plaintiffs' counsel says they are not seeking reimbursement for the purchase of the van itself but only for the modifications, which they submit were paid for by the deposit paid by Mr. Desbiens of \$34,849.60. He points out that Mr. Desbiens, prior to the car accident, was able to manage with a van with minimal modifications. They contend therefore that it was the injuries sustained in the car accident that necessitated the modifications to the Econoline van.

[314] The defendants submit that the WSIB paid for all the costs of the modifications to the Econoline van and that is compelling evidence that the modifications were necessary in order to accommodate Mr. Desbiens' paraplegia caused by the 1986 accident. They submit that the \$34,849.60 paid by Mr. Desbiens covered the cost of the van itself.

[315] I agree with this submission. At page 240 of Exhibit 10C there is a memo written by a Ms Litwiniok, an independent living consultant at the WSIB. The memo indicates that Mr. Desbiens met with her on July 24, 2002 and requested that the WSIB pay for the vehicle modifications for a new van. The memo further indicates that in 1987 the WCB paid for vehicle modifications to a 1987 Ford which included a wheelchair lift. Mr. Desbiens had this van until 1996 when he sold it and bought a used 1995 Caravan that did not have any modifications, as he was able to pull himself into the front seat and throw the wheelchair behind him. He then purchased a 1997 Caravan and was able to enter and exit the van in the same way that he did with the 1995 Caravan. The memo then goes on to state

worker is in receipt of a 100% PD for paraplegia and is entitled to vehicle modifications such as a modified van with a lift. He requires the modifications for wheelchair accessibility. He is now 60 years of age and even without the accident, was finding it more difficult to throw his wheelchair into the back.
(Emphasis added)

The memo concludes with Ms Litwiniok stating that she will make a submission to management for approval in principle. Mr. Desbiens was cross-examined on this memo. He recalled meeting with Ms Litwiniok, however, he denied making the statement underlined above. He maintains that he told her that his difficulty lifting the chair into the back was due to the car accident and that she said she didn't want to hear anything about the car accident because they wouldn't cover

anything arising out of the car accident, but if it didn't happen as a result of the car accident, he could buy the van and they would modify it for him.

[316] I think it is rather unlikely that Mr. Desbiens would have requested the WSIB to pay for the modifications and at the same time tell them, in effect, that he needed them because of the car accident rather than the 1986 accident. In any event, it is clear beyond all doubt that the WSIB recognized from the outset that it was obliged to pay for modifications to a van as a result of Mr. Desbiens' 1986 accident and did in fact pay for the modifications to the 1987 Ford. Even though the 1997 Caravan did not have as extensive modifications as the Econoline, it is quite clear that WSIB did not refuse to pay for the modifications; it was simply a case of Mr. Desbiens deciding he could do without them. I find that the amount paid by WSIB to Sparrowhawk was in fact for the modifications made to the van. I therefore conclude that the amount claimed by the plaintiffs is in fact for the van itself, which in my view is not a reasonable and necessary expense arising out of the car accident since he clearly needed a van before the car accident. I therefore disallow this claim in full.

Future Care

[317] This head of damages is by far the most significant and contentious. It will be helpful at the outset to briefly describe the witnesses and the documentary evidence relevant to this issue.

[318] The plaintiffs' main witness on this issue was Carol Kelly, an occupational therapist whose qualifications have been previously described.

[319] The plaintiff also called Robert Topping, a registered architect who is also a partner in DEI. His area of expertise is the designing of environments to accommodate persons with physical disabilities.

[320] Ms Kelly attended upon Mr. Desbiens at the request of his counsel on May 31, 2001 and June 6, 2001 for a period of approximately 10½ hours. On June 13, 2001 Mr. Topping attended at Mr. Desbiens' apartment to assess his home environment, take site measurements and photographs with a view to determining the deficiencies of Mr. Desbiens' living environment in

view of his disabilities. Ms Kelly and Mr. Topping then prepared a very detailed preliminary report dated June 27, 2001 outlining Mr. Desbiens, future care requirements including housing needs and their associated costs. On January 17, 2003 Ms Kelly again assessed Mr. Desbiens at his apartment. A supplementary future care report, dated March 15, 2003, as well as an updated Form 1 was prepared.

Housing

[321] Both Ms Kelly and Robert Topping testified about the deficiencies of Mr. Desbiens' existing housing accommodations in light of his impairments. A detailed list of these deficiencies is set out at page 46 of their June 2001 report filed as Exhibit 39. At page 66 of this report they identified 2 scenarios to respond to these deficiencies, namely the purchase of a condominium or a bungalow both of which would have to be renovated in order to accommodate Mr. Desbiens' impairments. The estimated costs of these scenarios are set out below:

Item	Cost
Condominium Scenario	
1. Purchase	\$227,000.00
2. Renovation	\$ 44,523.00
3. Moving expenses	\$800.00
4. Home maintenance	\$1,500.00
TOTAL	\$273,823.00
Bungalow Scenario	
1. Purchase	\$215,453.00
2. Renovation	\$110,957.00
3. Moving expenses	\$800.00
4. Home maintenance	\$3,000.00
TOTAL	\$330,210.00

[322] The report of March 5, 2003 report deals only to the bungalow scenario. The report indicates that pursuant to the direction of Mr. Neil Wheeler they were to research the costs that would be incurred by Mr. Desbiens were he to purchase and renovate a single story house to provide “functional, safe and dignified accommodation”. Accordingly Mr. Topping in conjunction with a real estate agent sought to identify suitable single story properties that could be renovated which met the following general criteria:

- Single story (bungalow) configuration – attached 1-1½ or 2 car garage;
- About 1500 sq. ft. of floor area (excluding the garage and any basement) or if smaller on a lot large enough to create approximately 1500 sq. ft. of floor area; and,
- Ideally located with the main floor close to grade on a lot that is generally level.

[323] On December 16, 2002 a MLS computer search identified only 4 potential properties that had listing prices ranging from \$324,888 – \$389,000.

[324] At page 15 of the March 5, 2003 report, the anticipated costs of renovations are estimated at \$169,550.00. The total costs of the house purchase and renovations and other expenses were estimated by Mr. Topping to be \$528,144 as set out in the table below.

No.	Item	Initial Purchase/One Time Cost	Replacement Frequency/ Years	Annual Cost
1.	House Purchase (average of listings)	\$356,944.00		
2.	House Renovations	\$169,550.00		
3.	Moving Expenses	\$1,650.00		
4.	Home Maintenance		1	\$3,000.00
	Total Costs	\$528,144.00		\$3,000.00

[325] In deciding how to best satisfy the housing requirements of Mr. Desbiens, Mr. Topping stated that he was guided by a consideration of what was economical and realistic. If the injured person’s existing accommodations were adaptable then he would propose adapting those

accommodations as required. Mr. Desbiens advised him that the landlord was not amenable to changes being made to the apartment. In any event, Mr. Topping was of the opinion that the existing apartment was not capable of being adapted because it was too small, the washroom facilities were inadequate and it lacked underground parking that was accessible by wheelchair.

[326] When asked why he was no longer considering the condominium scenario he indicated that a single storey home was preferable from a safety perspective because there is direct access from the grade level. I have some difficulty accepting this explanation. It seems clear that he restricted his March 5, 2003 report to the bungalow scenario because he was instructed to do so by Mr. Wheeler.

[327] He said he did not investigate appropriate rental accommodations because he understood that there were no apartments that were wheelchair accessible although he admitted that he had not done a search of the market for this kind of an apartment for 3 or 4 years.

[328] He did not consider a rental accommodation that might be suitable for renovation because he understood that landlords were reluctant to permit mechanical or structural changes. He admitted that he had not done any investigation into the question for a number of years. He also admitted that he had never investigated the availability of rental accommodations in Mississauga. He conceded that the current vacancy rate was the highest that it has been in many years and that landlords were very anxious to attract tenants.

[329] The defendants called Ms Pat Walker, Vice-President of Briar Lane Property Management Inc., the property manager of Mr. Desbiens' apartment building since September 1, 2002. It is her responsibility to approve any request for apartment modifications. Such requests must be made by letter and the tenant upon vacating the apartment must return the apartment to its original state. As long as there are no major structural changes such as the moving of a weight bearing wall and the modifications are in compliance of the building code or by-laws the request will be granted. She has received no request for modification to Mr. Desbiens' apartment since Briar Lane has managed the building. She was shown the quotation prepared at the request of the WCB dated September 18, 1986 for the proposed alterations to apartment 209

(Exhibit 40). She confirmed that if she had been requested to consent to the changes set out in the quotation, she would have done so.

[330] She testified that Briar Lane is the property manager for about 8800 apartment suites, mostly in the greater Toronto region. She stated that the current vacancy rate on average is 5% and that it is a tenant's market.

[331] The defendants submit that the type of modifications required by Mr. Desbiens are essentially the same as those modifications that were approved by the WCB in 1986 and that need for these modifications was created not by the car accident but by the 1986 accident. Alternatively, the defendants' submit that if the car accident caused the need for any modifications the most appropriate way to address these needs would be to locate a suitable rental accommodation or one that could be modified to accommodate Mr. Desbiens impairments caused by the car accident.

[332] The defendants also point out that Mr. Desbiens had made no effort to make any changes to his housing accommodations since the car accident even though he settled his SABS claims in June 2002 and, according to Ms Walker, a request for modifications along the lines approved by the WCB in 1986 would have been granted by the landlord. They asked the court to find that it was likely that the WCB would have funded the cost of the modifications but that in any event, Mr. Desbiens failure to modify the apartment amounted to a failure to mitigate his damages.

[333] The plaintiffs respond by pointing out that the defendants did not call any expert witness to challenge the reasonableness of the plaintiffs' housing scenario or to establish that appropriate rental accommodations exist for Mr. Desbiens. The plaintiffs submit that the bungalow scenario would be the best choice for Mr. Desbiens and that the condominium scenario identified in the 2001 report would be the next best housing option.

[334] Mr. Topping was of the opinion that in June 2001 the costs of modifications of a condominium at 1400 sq. ft. to Mr. Desbiens' requirements was \$44,523. In my opinion some of

these costs would have been related to modifications that were necessary as a result of Mr. Desbiens' impairments caused in the 1986 accident. On the other hand it did not include the costs of restoration in the event that Mr. Desbiens vacated the condominium. Doing the best I can with the evidence before me, I find that the reasonable costs of modifying a rental apartment to address the needs caused by the car accident and restoring it to its previous condition on vacating would be \$45,000. In addition I would allow \$800.00 as reasonable moving expense. I therefore assess the damages for housing costs in the amount of \$45,800.

Attendant Care

[335] After her 2001 assessment, Ms Kelly prepared an assessment of attendant care needs (Form 1) which follows the format used at a DAC to assess the attendant care needs of a claimant for SABS. Form 1 breaks down the attendant care requirements into 3 general categories - Level 1 - routine personal care, Level 2 - basic supervisory functions and Level 3 - complex health/care and hygiene functions. In each category the care requirements of the patient for each activity are listed as well as the assessor's estimates of the time required to perform each activity and the number of times each week it should be performed. Part 4 of Form 1 contains a calculation of the monthly attendant care costs of the each levels of care. In addition to Form 1, Ms Kelly prepared an attendant care assessment report ("ACAR") in which she provided in narrative form an explanation of the estimates in Form 1.

[336] On January 17, 2003 Ms Kelly again assessed Mr. Desbiens at his apartment. A supplementary ACAR dated March 15, 2003 as well as an updated Form 1 was prepared.

[337] On January 7, 2004 Ms Kelly again assessed Mr. Desbiens at his apartment and prepared a further supplementary future care report dated January 15, 2004. No changes were made to the ACAR or Form 1 dated March 15, 2003.

[338] On June 30, 2000 Mr. Desbiens, Ms Desbiens and Ms Norlock attended at the Assessment Center of West Park Hospital at the request of AXA, the SABs insurer of Mr. Desbiens for an Attendant Care Needs DAC ("ACDAC"). Kathy Blaney an occupational

therapist and Julie Curitti, a registered nurse carried out the assessment. An ACDAC report together with a Form 1 dated June 30, 2000 was prepared and forwarded to AXA insurance. I will refer to these documents as the West Park ACDAC report and the West Park Form 1 respectively. At trial, defendants' counsel attempted to file both the West Park ACDAC report and the Form 1 as business records pursuant to section 35 of the *Evidence Act*.⁵⁴ Plaintiffs' counsel indicated that he did not object to the Form 1 being filed as a business record, however, he submitted that the West Park DAC report was not a business record within the meaning of section 35. Accordingly Form 1 was marked as an exhibit, but the ACDAC report was not.

[339] However, I did allow counsel for the defendants, to cross examine Mr. Desbiens about the contents of the report insofar as they related to the types of activity observed by the assessors or statements made by Mr. Desbiens to the assessors during the assessment.

[340] Counsel for the defendants also cross-examined Ms Kelly on certain statements in the West Park DAC report. She accepted some as accurate and some as not.

[341] After hearing the submissions of the parties I granted leave to the defendants to call Ms Blaney for the limited purpose of testifying as to those aspects of the West Park DAC report that were not accepted as accurate by Mr. Desbiens and/or Carol Kelly.

[342] Ms Kelly's testimony with respect to attendant care needs was originally based on her report of January 17, 2003. As a result of hearing Ms Kelly's testimony it was obvious to me that a significant portion of Mr. Desbiens' attendant care needs resulted from the unsuitable nature of his living accommodations. In response to a question from the bench Ms Kelly confirmed that this was so. I therefore requested that Ms Kelly assess the attendant care needs on the basis that Mr. Desbiens was living in an appropriate wheel chair accessible environment modified to accommodate his impairments. In response to this request Ms Kelly prepared a revised Form 1, dated March 11, 2004, which was filed as Exhibit 52 at trial ("revised Form 1"). In the course of her further testimony she further amended two items of the revised Form 1, namely, extra laundering and hygiene.

⁵⁴ R.S.O. 1990, c. E.23, s. 35.

[343] The following chart summarizes the estimates provided by Ms Kelly of Mr. Desbiens' attendant care needs on the basis of her report of January 17, 2003 and her revised Form 1 and the estimates contained in the West Park Form 1 dated June 30, 2000.

Kelly report Jan 17, Kelly Revised West Park
2003 Form 1 Mar. Form 1
11 2004)

Level1	Min Per Week	Min Per week	Min Per Week
Dress & undress	420	210	140
Orthotics	105	105	105
Grooming	357	0	75
Feeding	140	42	140
Mobility	210	210	210
Extra Laundering	729	70*	105
Level 2			
Hygiene	329	90*	245
Overnight Security	3710	3710	0
Meals	300	70	420
Laundry	300	70	75
Housekeeping	310	140	105
Level 3			
Genito Urinary	791	420	420
Bowel Care	315	315	210
Exercise	140	140	35
Skin Care	290	245	105
Bathing	385	210	420
Maintenance Supplies Etc	105	70	36.25
Minutes per week	8831	5887	2810
Hours per week	147.18	98.12	46.83

Overnight Security

[344] By far the largest component of the attendant care in both of Ms Kelly's reports was overnight security. Ms Kelly was of the opinion that Mr. Desbiens was dependent on others for transfers to and from the wheelchair to his bed. She stated that this places him at risk overnight in the event of a fire or other emergency. In Ms Kelly's opinion an attendant must be available

to transfer him as required. Mr. Desbiens sleeps for 9 hours, or 540 minutes, per day. However, 10 minutes of the time has been allotted for the 3:00 a.m. catheterization, which is included in the category of genitourinary, resulting in a weekly allowance for overnight security of 3,710 minutes. Ms Kelly indicated that this allocation would be required regardless of the living arrangements of Mr. Desbiens. She also indicated she would be surprised if a reputable occupational therapist disagreed with the proposition that overnight care was required based on the observations she made.

[345] The West Park Form 1 allows nothing for overnight security. Ms Blaney testified that when she assessed Mr. Desbiens on June 30, 2000, she observed that he was able to transfer to and from the bed without assistance. She described in minute detail the manner in which Mr. Desbiens was able to transfer from the wheelchair to the bed and vice versa. She noted however, that in transferring from the wheelchair to the bed Ms Desbiens assisted him in positioning his right leg on to the bed.

[346] Ms Kelly on the other hand in her report of June 27, 2001 made the following observation:

To transfer to the wheelchair, Mrs. Desbiens stands in front of the wheelchair with her left foot placed beside the front caster to stabilize the chair. Mr. Desbiens is able to reach across to the right armrest with his right hand while lifting his body with his left hand to clear the wheel. Simultaneously, Mrs. Desbiens must bring his legs down from the bed and place them onto the footrests. Once Mr. Desbiens is in his wheelchair, he is able to reach the armrest to put it back onto the left side of the chair.

[347] In her report of March 5, 2003 Ms Kelly made the following observation:

Mr. Desbiens was observed transferring to his bed. His wife continues to lift and position his lower extremities on and off the bed for him. He is experiencing more problems lifting his buttocks to clear the tire of his wheelchair when transferring laterally to his left to the bed. He is now scraping his buttocks over the wheel rather than cleanly lifting himself using his upper extremities up and over the wheel. He is having further difficulty controlling the speed that he lowers himself when he transfers back into his wheelchair. The strength in his

upper extremities appears to have diminished when compared to the 2001 home visit.

[348] The plaintiffs submit that Ms Kelly's evidence should be accepted. They point out that Ms Kelly saw Mr. Desbiens on a number of occasions in 2001 and 2003 whereas Ms Blaney saw him only once in 2000. I must agree with the plaintiffs' submission. Ms Kelly's evidence in this regard was not shaken on cross-examination. Even if Ms Blaney's observations in 2000 were accurate, that does not necessarily mean that Ms Kelly's observations were inaccurate. Ms Kelly testified that Mr. Desbiens condition appears to have been deteriorating from 2001 to 2004.

[349] Even on Ms Kelly's evidence the assistance that an overnight attendant would be required to offer Mr. Desbiens for transfers is not substantial either in terms of time or physical effort. Indeed it is most likely that this attendant would not be required to do anything more than be available in case of an emergency. However, Ms Kelly was of the opinion that a personal care worker at an hourly rate of \$20.80 was required to provide this overnight security. This would result in an annual cost of approximately \$66,879. Thus the overnight security component of Mr. Desbiens' attendant care needs is greater than all the other components put together. I must confess that the expenditure of this amount of money to guard against the occurrence of an emergency, which on the evidence has never occurred in the 18 years that Mr. Desbiens has lived in the apartment as a paraplegic, seemed to me to be grossly excessive. In the defendants written submissions they address the overnight security allowance as follows:

There is an obligation upon the Plaintiff to mitigate his damages. The suggestion that the overnight attendant must be available is not reasonable. The Property Manager testified as to the Fire Safety Plan. Elements can be put into place to compliment such plan, such as the next door tenant or other such common-sense, practical option. Were such put into place, it is respectfully submitted that an Occupational Therapist undertaking an Attendant Care Assessment would not include a requirement for such Attendant Care needs in his or her Form 1.

[350] I agree that there probably are other "common sense practical options" to address the question of overnight security however the defendants failed to adduce any evidence of what they may be. It is true that Ms Walker testified that the apartment building has a priority list as

part of its fire safety plan. If Mr. Desbiens requested he could be placed on this list indicating the kind of assistance he requires. If the fire department attends the premises they look at this list to determine which tenants need special assistance. The defendants called no expert evidence to suggest that this adequately meets the overnight security needs of Mr. Desbiens. Furthermore, the defendants' own witness, Ms Blaney, testified that if she had determined that Mr. Desbiens could not transfer in and out of bed independently, she would have made an allowance for overnight security in the same manner as Ms Kelly.

[351] In *Dube v. Penlon*, *supra*, the court was faced with a similar problem. That case dealt with an injured child who was totally dependant on others. The plaintiff had sought an hourly rate of \$17.25 for overnight attendant care. Zuber J. found that this was a task that could be performed by a student at minimum wage and fixed the hourly rate at \$6.75. I propose to adopt the eminently reasonable approach of Zuber J. I therefore allow the attendant care for overnight security estimated by Ms Kelly at the hourly rate of \$8.00, which, I understand, is approximately the minimum wage for domestic workers in Ontario.

Other Attendant Care Needs

[352] With respect to the other attendant care needs as estimated by Ms Kelly, as set out in her Revised Form 1, in the absence of any evidence to the contrary, I find them to be a reasonable and necessary consequence of the car accident and I allow them in full.

Other Activities of Daily Living Costs

[353] This claim is summarized in the chart below.

No.	Item	Initial Purchase/One Time Cost	Replacement Frequency/ Years	Annual Cost
1.	Overbed Table	\$267.00	10	\$26.70
2.	Long Handled Reacher	\$20.00	3	\$6.67

3.	Rehab Shower Commode	\$1,280.00	5	\$256.00
4.	Repairs & Maintenance of Shower Commode		1	\$100.00
5.	Long Handled Bath Sponge	\$6.00	1	\$6.00
6.	Housekeeping		1	\$19,968.00
7.	Home Maintenance – Apartment			1,500
9.	Assistive devices	\$250.00	1	\$150.00

Overbed Table

[354] Ms Kelly testified that this was required in order to keep supplies nearby and to keep a working surface for the bed. I allow this claim.

Long Handled Reacher

[355] Ms Kelly testified that this was required in order to provide access to items at high and low levels. I allow this claim.

Rehab Shower Commode

[356] Ms Kelly testified that this was required in order to ensure safety in the shower and to allow independent mobility in the bedroom. As I understand her evidence, this cost is to replace the commode that is provided by the WSIB with a higher end model. I am not satisfied that this is a necessary and reasonable item. I disallow this claim as well as the claim for Repairs and Maintenance.

Long Handled Bath Sponge

[357] I allow this claim.

Housekeeping

[358] The plaintiffs claim that this head of damages covers the housekeeping tasks that Mr. Desbiens would have performed had the car accident not happened. For the reasons that I have given in dealing with the claim for special damages for housekeeping I would allow this claim in the amount of 18 hours per week at an hourly rate of \$12.00 per hour. I therefore allow \$11,232 per year.

Home Maintenance – Apartment

[359] The plaintiffs claim that prior to the car accident Mr. Desbiens was able to perform a number of minor home maintenance services such as painting, repairing screen doors etc. and that as a result of the car accident he is no longer able to do so. I find that an annual amount of \$1,500 is excessive. I will allow the sum of \$500.00 per year for this claim.

Assistive devices

[360] This claim is for an initial allowance for the purchase of assistive devices and thereafter, an annual allowance for the purchase of replacement or new devices to assist with activities of daily living. I allow it.

Mobility

[361] This claim is summarized in the chart below.

No.	Item	Initial Purchase/One Time Cost	Replacement Frequency/ Years	Annual Cost
1.	Terminator Jr. Wheelchair	\$5,200.00	5	\$1,040.00
2.	Repairs & Maintenance		1	\$500.00
3.	Jay Triad Cushions (2)	\$580.00	2	\$290.00
4.	Extra Cushion Covers (2)	\$170.00	2	\$85.00
5.	Infinity Low Back Support	\$500.00	2	\$250.00
6.	Repairs & Maintenance, seating		1	\$75.00
7.	Power Wheelchair	\$10,000.00	5	\$2,000.00
8.	Batteries (2)	\$450.00	2	\$225.00
9.	Repairs & Maintenance (power)		1	\$1,000.00
10.	King Electric Split Bed	\$3,110.00	20	\$155.50

11.	Action Gel Pad (\$1,100.00)		5	\$220.00
12.	Lounge Chair	\$1,295.00	15	\$86.33
13.	Repairs & Maintenance, general		1	\$150.00
14.	Wheelchair bag	\$40.00	2	\$20.00

[362] I allow Item 10. This is the cost of a king size electric bed which allows independent positioning of each side of the bed since this appears to be related to the injuries sustained in the car accident. However, I am not satisfied that the need for the balance of the items was caused by the injuries sustained in the car accident but rather by Mr. Desbiens' pre-existing paraplegia. Accordingly I disallow them.

Transportation

[363] This claim is summarized in the chart below.

No.	Item	Initial Purchase/One Time Cost	Replacement Frequency/ Years	Annual Cost
1.	Cellular Package		1	\$120.00
2.	Motor League, year one	\$109.16		
3.	Motor League, annual		1	\$94.16
4.	Neck Rest	\$8.02	3	\$2.67
5.	Modified Van	\$75,538.40	7	\$10,791.20
6.	Mileage		1	\$1,200.00

[364] Item 5 - The plaintiffs assert this claim essentially on the same basis as the claim for vehicle modification under the head of special damages except that here they appear to be claiming the costs of the van as well as the modifications. As previously stated I find that the need for the van was not a consequence of the car accident since Mr. Desbiens needed a van prior to the car accident due to his paraplegia. Insofar as the modifications are concerned, I reject this part of the claim for the same reasons as I rejected the special damage claim.

[365] Item 1,2,3,6- I fail to see how the need for these items is related to the car accident. They are disallowed.

Rehabilitation

[366] This claim is summarized in the chart below.

No.	Item	Annual Cost
1.	Occupational Therapy	\$480.00
2.	Foot care	\$405.00
3.	Case Management	\$2,500.00

[367] I allow items these items in full.

Life Skills

[368] The plaintiffs claim the sum of \$1,000 for an adjustable workstation which I allow in full.

Medications

[369] This claim is summarized in the chart below.

No.	Medication	Dosage	Frequency	Cost	Annual Cost
1.	Paxil	20 mg	1 tablet/day	\$71.18 (30 pills)	\$865.05
2.	Lorazepam	1 mg	1 tablet 3 x daily	\$16.15 (100 pills)	\$175.20
3.	Mobicox	7.5 mg	1 tablet daily	\$40.42 (30 pills)	\$270.10
4.	Tylenol #3		6.5 tablets daily	\$26.51 (200 pills)	\$308.49
5.	Metamucil	2 Tbsp.	Daily	\$34.21 (2 bottles)	\$410.52

[370] WSIB continues to pay half of the cost of the Tylenol #3 and Lorazepam. WSIB also pays 100% of the cost of Metamucil. I allow 50% of the claim for Tylenol #3 and Lorazepam and 100% of the claim for Paxil and Mobicox.

Nerve Blocks and Botox Treatments

[371] Dr. Greenspoon recommended that Mr. Desbiens be given injections with Botox, which has been used for some time for cosmetic purposes, but it is a relatively new type of treatment for chronic pain. Botox is essentially a poison that paralyzes the muscles in the area where it is injected. Dr. Greenspoon stated that there were approximately 30 studies on the subject of using Botox for chronic pain and more were ongoing. This use of Botox is not yet approved by OHIP or by the F.D.A. in the U.S.A. He conceded that Botox was experimental for headaches but he has been using it on patients for about two years on patients with migraines.

[372] Dr. Greenspoon explained that if muscle spasm is the cause of the pain, Botox might be very successful. Thirty to forty percent of patients report significant relief. Dr. Greenspoon stated that the great advantage of Botox is that treatment is required only once every 3 months, sometimes only 2 or 3 treatments are needed and sometimes it continues on longer. Usually you know if it works after the first treatment. Although the studies suggest a frequency of once every 3-6 months, because Mr. Desbiens pain is chronic he might need it more frequently, perhaps once every 3 months on an indefinite basis.

[373] Dr. Greenspoon has not tried Botox on Mr. Desbiens because it is very expensive. One treatment costs from \$1,200 - \$1,600 and neither OHIP nor insurance will pay for the treatment. The plaintiffs claim the cost of these proposed treatments. They also claim for the future cost of the nerve blocks on the basis that these treatments may in the future not be covered by OHIP.

[374] The defendants submit that the benefits of the proposed Botox treatment are speculative. They point out that Mr. Desbiens recovered substantial moneys from the SABS settlement in June 2002 and has had the funding to undertake the Botox treatments or at least to undertake a trial for such treatments. They submit that the cost of Botox treatments ought not to

be recoverable against the Defendants. They also submit that there is no evidence to support the contention that the nerve block treatment provided by Dr. Greenspoon, which is now covered under OHIP, will be de-listed at some point within Mr. Desbiens life expectancy. I agree with these submissions. I disallow this claim.

Discount Rate

[375] Rule 53.09 of the *Rules of Civil Procedure* sets out the prescribed discount rate for calculation of awards for future pecuniary damages. The prescribed rate is 2.25% for the first 15 years and 2.5% thereafter. However, the court has the discretion to depart from the prescribed discount rate when it is justified by the evidence.⁵⁵

[376] The plaintiffs' called Peter C. Coyte, who is an economist with outstanding qualifications in the area of Canadian health care economics. The essence of Dr. Coyte's evidence was that the expected future care cost inflation for most health care costs in Ontario would be 3.6% per annum for the first 15 years and 3.25% per annum thereafter, whereas the expected general rate of inflation would be 3.0% per annum and 2.75% per annum respectively. To allow for this, the actuarial present value of the future care items in the activities of daily living schedule, excluding housekeeping costs, and in the mobility, rehabilitation, medication and supplies and future needs schedule of the DEI report and the items in the Greenspoon report, should appropriately be 1.65% for the first 15 years and 2.0% thereafter. This evidence was not seriously challenged on cross-examination nor did the defendants call any evidence to the contrary. I therefore accept Dr. Coyte's evidence with respect to the appropriate discount rates for health care costs.

Contingencies

[377] The defendants submit that the assessment for future care costs should be reduced by a significant percentage to allow for contingencies. The defendants submit that at the time of the car accident Mr. Desbiens had been a paraplegic for more than 13 years and that it is reasonable to expect that even if the car accident had not occurred that he probably would have required a

significant amount of the future care that is being claimed due to natural deterioration of his paraplegic condition with age.

[378] The plaintiffs maintain that there is insufficient evidence to support a deduction for specific contingency of deterioration based on his paraplegia. Moreover, they submit that the evidence suggests that Mr. Desbiens was functioning well before the accident and that it was impossible to predict the extent to which his health would have deteriorated if the car accident had not occurred. They point out that the defendants had attempted to introduce expert evidence to support their position through the occupational therapist Melody Lynden but the court had ruled that she did not possess the requisite expertise to express an opinion on the issue of what Mr. Desbiens future care needs would have been even if he had not been involved in a motor vehicle accident. The defendants made no further efforts to introduce this type of evidence.

[379] A court assessing future pecuniary loss is essentially called upon to answer the following questions: will the plaintiff as a result of the defendant's wrongful act suffer a loss in the future and, if so, what is the quantum of that loss? The manner in which the court should approach this task has been aptly stated in a decision of the House of Lords:

The role of the court in making an assessment of damages which depends on its view as to what will be and what would have been is to be contrasted with its ordinary function in civil actions in determining what was. In determining what did happen in the past a court decides on the balance of probabilities. Anything that is more probable than not it treats as certain. But in assessing damages, which will depend upon its view as to what will happen in the future or would have happened in the future if something had not happened in the past, the court must make an estimate as to what are the chances that a particular thing will or would have happened and reflect those chances, whether there are more or less than even in the amount of damages which it awards.⁵⁵

⁵⁵ *McDermid et al. v. Ontario et al.* (1985), 53 O.R. (2d) 495 (H.C.)

⁵⁶ *Mallett v. McMonagle*, [1970] A.C. 166 (H.L.). This was referred to as the correct principle in the assessment of pecuniary damages by the Supreme Court of Canada in *Naylor Group Inc. v. Ellis-Don Construction Ltd.* (2001), 204 D.L.R. (4th) 513 at 540.

[380] Factors affecting the degree of risk of future pecuniary loss and the possibility that some of those losses may have occurred in any event are referred to as contingencies.⁵⁷ It is the general practice to take contingencies into account by assigning a percentage factor that will reflect the degree to which such contingencies will reduce or in some cases increase the potential loss.

[381] There are both general and specific contingencies. Here the defendants are relying on a specific contingency, that is, one that applies specifically to Mr. Desbiens. A party who relies on a specific contingency, whether it is a positive or a negative contingency, must be able to demonstrate that the evidence supports the real possibility of the contingency occurring.

[382] The defendants submit that there is sufficient evidence to support the real possibility of the contingency on which they are relying. They submit that it is common sense and reasonable to expect that there would be deterioration in the state of health of a paraplegic as he grows older and that the longer an individual has been a paraplegic, the greater the likelihood of deterioration. They refer to the following passages from Dr. Ameis evidence:

Deterioration is common in paraplegia, on the basis of aging, injury or illness. The devolution into dependency upon an electric wheelchair is one of the common consequences of these forms of deterioration. Therefore I would regard the need of an electric wheelchair as ultimately falling into the domain of responsibility of the WSIB since the paraplegia was caused in a work accident.

The patient that you see at age 20 who had fallen out of a tree or dived into an empty pool and developed paraplegia is not going to be the same patient that you're managing when he enters into his 40s, 50s, 60s, 70s because wear and tear is accelerated, magnified in a paraplegia, because any form of injury or any form of illness is capable of magnifying the situation. And cumulatively as you get to the older years, you are therefore treating a different person. So an individual in his fifties may have a minor heart problem which would not be a relevant issue if he was a normal individual, but because a paraplegic is exerting so much more energy to do normal activities that minor heart problem now becomes a major impairment, a major disability, a source of disability. So my point is that an individual who starts out with a manual wheelchair and in later years needs an

⁵⁷ *Graham v. Rourke* (1990), 74 D.L.R. (4th) 1 at 13 (Ont. C.A).

electric wheelchair is completely within the expectation for my practice and I think that of every other physiatrist.

[383] Ms Kelly also acknowledged that it is reasonable as time goes by paraplegics could deteriorate and that sometimes the rate is faster and that sometimes it is slower.

[384] Dr. Finlayson expressed the opinion that if Mr. Desbiens undertook a multidisciplinary chronic pain management program there was a 25% chance of substantial improvement in his activities of daily living.

[385] The plaintiffs submit that Dr. Ameis' evidence relates only to the issue of Mr. Desbiens need of an electric wheelchair and there was nothing in any of Dr. Ameis reports that addressed the issue of the general deterioration of Mr. Desbiens' condition and his increased need for future care if the accident had not occurred. While Dr. Ameis' evidence was directed specifically to the issue of the electric wheelchair it does provide support for the general proposition that that as a paraplegic ages it is reasonable to expect some deterioration in his condition that will necessitate additional assistive equipment and care. It would, of course, have been preferable for the defendants to have called some expert evidence to address this issue but for some inexplicable reason they attempted to do through a witness who had no expertise in the matter.

[386] Nonetheless, I am satisfied that the evidence before me does support the real possibility that, as Mr. Desbiens grew older, there would have been some deterioration in his condition that would have necessitated some of the future care claimed even if the car accident had not occurred. In my opinion a 15% reduction in the assessment of the future care costs for contingencies is appropriate.

Life Expectancy and Actuarial Evidence

[387] The parties agreed that Mr. Desbiens' future life expectancy is 14.75 years from April 16, 2004 and that the appropriate present value (PV) factors are set out in the report of the actuary Thomas Schinbein dated April 22, 2004, filed as ex. 70. The present value of \$1,000 per year in respect of health care costs based on Professor Coyte's evidence is \$12,780 and in respect of other costs is \$12,159.

Summary of Future Care Costs

[388] The chart below summarizes my findings in respect to future care costs. It should be noted that I have applied the present value factor for health care costs to all items except housekeeping and overnight security. I find that professor Coyte's reasoning would not apply to the type of personnel that I have found to be appropriate to fulfil the role of overnight security.

	One time	Annual	PV factor	TOTAL Present Value
Housing	\$40,800.00			
Attendant Care				
Attendant care without OS		\$40,505.85	12.78	\$517,664.76
Overnight Security		\$25,722.69	12.159	\$312,762.19
Activities of Daily Living				
Overbed Table	\$267.00	\$26.70	12.78	\$215.00
Long Handled Reacher	\$20.00	\$6.67	12.78	\$75.00
Long Handled Bath	\$6.00	\$6.00	12.78	\$80.00
Sponge				
Assistive Devices	\$250.00	\$150.00	12.78	\$1,917.00
Home Maintenance		\$500.00	12.78	\$6,079.50
King Electric Split Bed	\$3,100.00	\$0.00	12.78	
Housekeeping		\$11,232.00	12.159	\$136,569.89
Rehabilitation				
Occupational Therapy		\$480.00	12.78	\$6,134.40
Footcare		\$405.00	12.78	\$5,175.90
Case management		\$2,500.00	12.78	\$31,950.00
Life Skills	\$1,000.00		12.78	
Medications and Supplies				
Paxil		\$865.05	12.78	\$11,055.34
Lorazipam		\$87.60	12.78	\$1,119.53
Mobicox		\$270.10	12.78	\$3,451.88
Tylenol		\$154.25	12.78	\$1,971.32
TOTAL	\$45,443.00			\$1,036,221.70
TOTAL One Time and				\$1,081,664.70

PV**TOTAL After
Contingency Applied
(15%)****\$919,415.00****Deductions for SABs Received**

[389] Section 267.8 (4) of the *Act* requires the court reduce the damages for health care expenses by any amount received or available before the trial for SABs in respect of health care expenses. Section 267.8 (6) requires the court to reduce the damages for other pecuniary loss by any amount received or available before trial for SABs other pecuniary loss. The parties have agreed that the following amounts have been received by Mr. Desbiens for SABs.

Medical Rehabilitation:	\$100,000
Attendant Care:	\$59,542
Housekeeping:	\$10,400

[390] Therefore the sum of \$159,542 should be deducted from the award for damages for health care expenses. The sum of \$10,400 should be deducted from the award for damages for Housekeeping expenses. Since I have made awards under these heads of damages in excess of the SABs received. I propose to make a global reduction. If the parties feel that a more specific allocation is necessary and cannot agree on how it should be done, I may be spoken to.

Summary of Damage Assessment

General Non-Pecuniary (less statutory deductions) -	\$135,000
FLA Claims -	\$77,500
Specials -	\$56,756
Future Care Costs -	\$919,415.
Deductions for SABS Paid -	(\$169,992)
TOTAL -	\$1,018,729

Gross-up

[391] Section 267.10 of the *Insurance Act* indicates that in the circumstances prescribed by the regulations, the court shall order that an award for damages in an action for loss or damage from bodily injury or death arising directly or indirectly from the use or operation of an

automobile shall be paid periodically on such terms as the court considers just. The relevant regulation is Ontario Regulation 461/96. Section 6 of this regulation outlines the circumstances in which an award for damages for pecuniary loss shall be paid periodically. Section 116 of the *Courts of Justice Act* must also be considered. The parties have agreed that the issue of whether any portion of the award is to be structured whether a gross-up applies can be determined after the parties have received the Court's reasons and have made submissions thereon.

[392] The plaintiffs are entitled to pre-judgment interest in accordance with the provisions of the *Courts of Justice Act*.

[393] Counsel may contact my secretary with respect to setting up a conference call or appointment to deal with any mathematical or typographical errors in these reasons and to speak to the matters of costs, gross-up, and pre-judgment interest or any other matter that is required to translate these reasons into a formal judgment.

[394] I am obliged to counsel for their very helpful oral and written submissions.

H. SPIEGEL J.

Released: November 17, 2004

COURT FILE NO.: 00-CV-185811

DATE: 20041004

ONTARIO

SUPERIOR COURT OF JUSTICE

B E T W E E N:

PHILLIPE DESBIENS and CECILE DESBIENS

Plaintiffs

- and -

MICHAEL MORDINI and MARIA LUONGO

Defendants

REASONS FOR JUDGMENT

H. SPIEGEL J.

Released: November 17, 2004